RBT TEST CONTENT OUTLINE, 3RD EDITION



RBT Training Profiles

One page for every task in the BACB RBT Test Content Outline (3rd ed.): definitions, procedure, examples, and exam tips.

A Data Collection and Graphing

B Behavior Assessment

C Behavior Acquisition

D Behavior Reduction

E Documentation and Reporting

F Ethics

79 training profiles across 6 domains and 43 tasks. Not affiliated with, endorsed by, or sponsored by the Behavior Analyst Certification Board.

About This Guide

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Sources

This guide is organized by the BACB RBT Test Content Outline (3rd ed.) and uses the BACB RBT Ethics Code (2.0). These documents are the official sources and are available at bacb.com. Check bacb.com and the current RBT Handbook for the latest requirements, which can change.

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Not clinical or legal advice

This guide supports exam study. It is not a substitute for your supervisor, your employer's policies, a client's behavior plan, or legal advice. RBTs follow the written plan and their supervisor's direction.

Use

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How to Use This Guide

Each page covers one concept from the outline. Every profile has the same five sections, so you always know where to look.

1. **Task code and title**
   The outline task and the concept name, with a core definition.
2. **Key concepts and terminology**
   Essential terms and the nuances that separate look-alike concepts on the exam.
3. **Step-by-step procedure**
   Four steps, plus the RBT scope boundary: what the RBT does and what the BCBA decides.
4. **Real-world scenario**
   An example and a non-example, with the reason the non-example is wrong.
5. **Exam tip and high-yield summary**
   The key words that point to the correct answer.

Study method

1. Read one profile. Cover sections 2 to 5 and say the definition from memory.
2. Write your own example and non-example. Compare them with section 4.
3. Study look-alike pairs together, such as latency and interresponse time, or extinction burst and resurgence.
4. Weight your time by exam share. Domain C and Domain D carry the most questions.
5. Always check the scope boundary. Many questions test who decides: the RBT or the BCBA.

Notation: S^D is a discriminative stimulus. BCBA is the supervisor who writes and changes the plan.

Exam Blueprint

The RBT examination has 75 scored questions and 10 unscored pilot questions. The outline has 43 tasks in 6 domains.

#	DOMAIN	TASKS	QUESTIONS	SHARE		PAGE
A	Data Collection and Graphing	8	13		17%	5
B	Behavior Assessment	3	8		11%	21
C	Behavior Acquisition	11	19		25%	30
D	Behavior Reduction	7	14		19%	52
E	Documentation and Reporting	4	10		13%	74
F	Ethics	10	11		15%	79
	Total	**43**	**75**		**100%**	

Where to spend your time

Domain C (19 questions) and Domain D (14 questions) together make up 44% of scored questions. Domain A is next at 13 questions. Check bacb.com for the current outline in case question counts change.

DOMAIN

A

Data Collection and Graphing

13 QUESTIONS · 17% OF EXAM

8 TASKS · 15 PROFILES

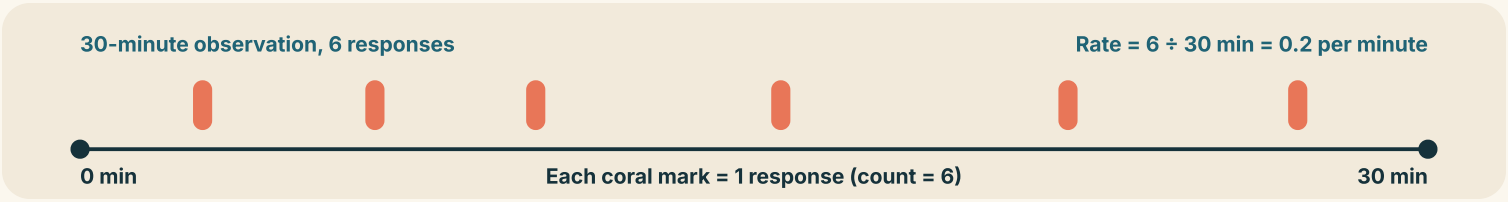
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TASK
A.1

1 TASK A.1 · CONTINUOUS MEASUREMENT

Frequency

Core definition. A count of the number of times a behavior occurs. Dividing the count by the observation time gives rate.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Count
The number of complete instances of the target behavior observed.

Observation period
The fixed amount of time during which every instance is counted.

Rate
Count divided by observation time (e.g., 6 per 30 minutes = 0.2 per minute).

Discriminative nuances

Frequency vs. Rate
Frequency is the raw count. Rate adds time, and is needed to compare sessions of different lengths.

Frequency vs. Duration
Frequency fits behavior with a clear start and end and a similar length each time (hand raising). Long episodes such as tantrums call for duration.

3 STEP-BY-STEP PROCEDURE

1. **Review the definition**
   Confirm exactly what counts as one instance, and what does not.
2. **Start the observation period**
   Note the start time or start the timer, then begin observing.
3. **Tally each instance**
   Make one tally as each complete instance occurs. Do not tally an ongoing instance twice.
4. **End and record**
   Stop at the end of the period. Record the total count and the observation time.

RBT SCOPE BOUNDARY

RBTs are responsible for tallying, recording, and graphing frequency data. The BCBA selects frequency as the measure and defines the target behavior and observation period.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

During a 30-minute session, an RBT makes a tally each time the learner says “help” to request assistance. The tally total is 6.

Recorded: 6 responses in 30 minutes

NON-EXAMPLE **Common pitfall**

Incorrect: Writing “many” on the data sheet, or treating 8 responses in a 60-minute session as higher than 6 responses in a 30-minute session.

Why it’s wrong: Record exact counts. Different session lengths must be compared as rate: 6 ÷ 30 = 0.2 per minute vs. 8 ÷ 60 = about 0.13 per minute.

5 · EXAM TIP
High-yield summary

Test takeaway: Look for “how many times,” “number of occurrences,” or “per minute/hour.” If session lengths differ, convert to rate before comparing.

TASK
A.1

1 TASK A.1 · CONTINUOUS MEASUREMENT

Duration

Core definition. The total amount of time from the onset of a behavior to its offset (end).



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Onset
The moment the behavior begins, as stated in the operational definition.

Offset
The moment the behavior ends, as stated in the operational definition.

Clean timer
Reset the timer to 00:00:00 at the onset of each episode so each duration is read directly from the timer.

Discriminative nuances

Duration vs. Latency
Duration starts when the behavior starts. Latency ends when the behavior starts.

Duration vs. Frequency
Frequency counts how many times the behavior occurs. Duration measures how long each episode lasts.

3 STEP-BY-STEP PROCEDURE

1. **Confirm start and end rules**
   Know exactly what marks onset and offset before the session.
2. **Start the timer at onset**
   Start the instant the behavior begins, not when the instruction is given.
3. **Stop the timer at offset**
   Stop the instant the behavior ends. Restart for each new episode.
4. **Record each episode**
   Write down each episode's duration separately. Adding them gives total duration, a different measure.

RBT SCOPE BOUNDARY

RBTs are responsible for timing, recording, and graphing duration data. The BCBA decides when duration is the right measure and writes the onset and offset rules.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

The client begins crying at 00:00:00 and stops at 00:04:30 (4 min 30 s). For the second episode the RBT starts over with a clean timer at 00:00:00 when the crying starts, and stops it at 00:01:30 (1 min 30 s).

Durations: 4 min 30 s and 1 min 30 s

NON-EXAMPLE **Common pitfall**

Incorrect: Starting the timer when the RBT says "Start your worksheet" instead of when the learner begins working.

Why it's wrong: The time between the instruction and the first action is latency. Duration starts at behavior onset.

5 · EXAM TIP
High-yield summary

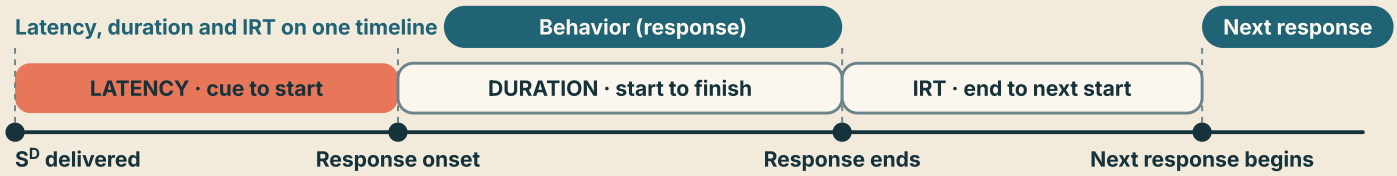
Test takeaway: Look for "how long," "length of time," or "time engaged." If the question asks about time before the behavior starts, it is latency.

TASK
A.1

1 TASK A.1 · CONTINUOUS MEASUREMENT

Latency

Core definition. The amount of time that elapses between the onset of a specific stimulus (such as an instruction or prompt) and the initiation of a behavior.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Discriminative Stimulus (S^D)
The cue or instruction given (e.g., "Put on your shoes").

Response Onset
The exact moment the client begins performing the target behavior.

Temporal Locus
The measurement dimension focused on when in time a behavior occurs relative to other events.

Discriminative nuances

Latency vs. Duration
Latency measures the time before the behavior starts (from cue to start). Duration measures the time from when the behavior starts to when it finishes.

Latency vs. IRT (Interresponse Time)
Latency is the gap between a stimulus and a response. IRT is the gap between the end of one response and the start of the next response.

3 STEP-BY-STEP PROCEDURE

1. **Prepare timer** — RBT holds 00:00:00
   Have a timer or stopwatch ready before delivering the instruction.
2. **Deliver S^D & start timer** — ▶ 00:00:01
   Deliver the clear instruction (e.g., "Open your book") and start the timer immediately as you finish speaking.
3. **Observe onset**
   Watch closely for the moment the client begins the movement/action.
4. **Stop timer & record**
   Stop the timer the instant the behavior starts. Record the elapsed time (e.g., seconds or minutes) on your data sheet.

RBT SCOPE BOUNDARY
RBTs are responsible for accurately collecting, timing, and graphing latency data. The BCBA determines when latency measurement is clinically appropriate and designs any systematic prompting strategies to decrease latency.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

While holding a timer at 00:00:00, an RBT says "Sit down, please" and clicks start on the timer to measure the time after the S^D is given to when the client starts to pull out their chair and begin to sit.

Recorded latency: 00:00:12

NON-EXAMPLE **Common pitfall**

Incorrect: Timing from when the RBT says "Sit down" until the learner's bottom actually hits the chair seat at 10:00:18 AM (18 seconds).

Why it's wrong: That 18-second total includes execution time (Duration). Latency stops the second the movement to sit begins.

5 · EXAM TIP
High-yield summary

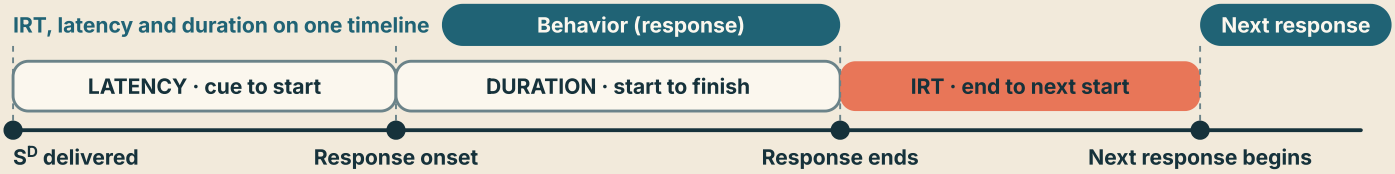
Test takeaway: Look for keywords like "time between prompt and start of behavior" or "how long it took them to start." If the scenario asks about the time to finish or the time between two behaviors, it is not latency.

TASK
A.1

1 TASK A.1 · CONTINUOUS MEASUREMENT

Interresponse Time (IRT)

Core definition. The amount of time that elapses between the end of one response and the start of the next response.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Response offset
The moment one instance of the behavior ends.

Next response onset
The moment the following instance of the behavior begins.

Relation to rate
Short IRTs mean fast responding and a high rate. Long IRTs mean slow responding and a low rate.

Discriminative nuances

IRT vs. Latency
IRT starts when a response ends. Latency starts when a stimulus is delivered.

IRT vs. Duration
IRT is the gap between two responses. Duration is the length of one response.

3 STEP-BY-STEP PROCEDURE

1. **Confirm clear start and end**
   The behavior must have a definite onset and offset so a gap can be timed.
2. **Start the timer at offset**
   Start the instant one response ends.
3. **Stop the timer at next onset**
   Stop the instant the next response begins.
4. **Record and repeat**
   Write down the elapsed time. Restart at the next offset.

RBT SCOPE BOUNDARY

RBTs are responsible for timing, recording, and graphing IRT data. The BCBA decides whether IRT is the right measure, such as when pacing of responses is the goal.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

The RBT starts the timer at 00:00:00 when the client finishes swallowing, and stops the timer at the start of the next bite, at 00:00:30.

IRT: 30 seconds

NON-EXAMPLE **Common pitfall**

Incorrect: Starting the timer when the first bite began, stopping it when the next bite began at 00:00:40, and recording 40 seconds.

Why it's wrong: That total includes the 10-second duration of the first bite. IRT starts at the offset of the first response, when the client finishes swallowing.

5 · EXAM TIP
High-yield summary

Test takeaway: Look for "time between two responses" or "time between bites/hits." A cue as the starting point means latency; the end of one response as the starting point means IRT.

TASK
A.2

1 TASK A.2 · DISCONTINUOUS MEASUREMENT

Partial Interval

Core definition. The observation period is divided into equal intervals. An interval is scored if the behavior occurs at any point during the interval.



Each box = 10 s · coral = behavior occurring

3 of 6 scored + = 50%

2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Interval

A short, equal block of the observation period (e.g., 10 seconds).

Scoring

Each interval is marked + (behavior occurred) or – (behavior did not occur).

Overestimation

Partial interval tends to overestimate how much behavior occurs.

Discriminative nuances

Partial vs. Whole Interval

Partial scores any occurrence. Whole scores only if the behavior lasts the entire interval.

Partial Interval vs. MTS

Partial requires watching the whole interval. MTS checks only the moment the interval ends.

3 STEP-BY-STEP PROCEDURE

1. **Set the intervals**
   Set the interval length and a cue (timer or vibration) for the end of each one.
2. **Observe the full interval**
   Watch the learner for the entire interval.
3. **Score the interval**
   Mark + if the behavior occurred at any point, even briefly. Mark – only if it did not occur.
4. **Calculate the percentage**
   Intervals scored + ÷ total intervals × 100.

RBT SCOPE BOUNDARY

RBTs are responsible for implementing interval recording and calculating the percentage. The BCBA chooses the interval type and length.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

Using 10-second partial intervals across 1 minute (6 intervals), an RBT sees hand flapping in intervals 1, 2, and 5 only.

3 of 6 intervals = 50%

NON-EXAMPLE **Common pitfall**

Incorrect: Leaving interval 1 blank because the behavior stopped after 4 seconds, before the 10-second interval ended.

Why it's wrong: Partial interval scores any occurrence during the interval, however brief. Interval 1 is scored +.

5 · EXAM TIP
High-yield summary

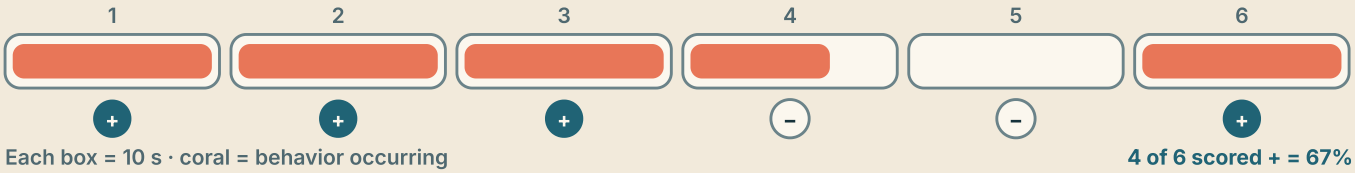
Test takeaway: "At any point during the interval" means partial. "Throughout the entire interval" means whole. "At the end of the interval" means MTS.

TASK
A.2

1 TASK A.2 · DISCONTINUOUS MEASUREMENT

Whole Interval

Core definition. The observation period is divided into equal intervals. An interval is scored only if the behavior occurs during the entire interval.



Each box = 10 s · coral = behavior occurring

4 of 6 scored + = 67%

2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Interval
A short, equal block of the observation period (e.g., 10 seconds).

Entire interval
The behavior must be present from the first second to the last second.

Underestimation
Whole interval tends to underestimate how much behavior occurs.

Discriminative nuances

Whole vs. Partial Interval
Whole is the stricter rule: a brief stop makes the interval −. Partial scores any occurrence.

Whole Interval vs. Duration
Whole interval gives a percentage of intervals, not the exact time. It is often used for behavior to increase, such as on-task.

3 STEP-BY-STEP PROCEDURE

1. **Set the intervals**
   Set the interval length and a cue for the end of each one.
2. **Observe the full interval**
   Watch the learner for the entire interval without looking away.
3. **Score the interval**
   Mark + only if the behavior was present the whole time. If it stopped at all, mark −.
4. **Calculate the percentage**
   Intervals scored + ÷ total intervals × 100.

RBT SCOPE BOUNDARY

RBTs are responsible for implementing interval recording and calculating the percentage. The BCBA chooses the interval type and length.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

Using 10-second whole intervals across 1 minute (6 intervals), an RBT records on-task behavior. The learner stays on task the entire interval in intervals 1, 2, 3, and 6.

4 of 6 intervals = 67%

NON-EXAMPLE **Common pitfall**

Incorrect: Scoring interval 4 + because the learner was on task for 7 of the 10 seconds.

Why it's wrong: Whole interval requires the entire interval. Seven of ten seconds is not enough, so interval 4 is −.

5 · EXAM TIP
High-yield summary

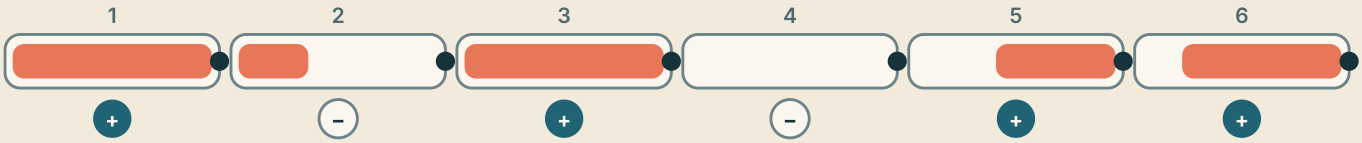
Test takeaway: "Throughout," "entire," or "the whole interval" points to whole interval. It underestimates behavior, and partial interval overestimates it.

TASK
A.2

1 TASK A.2 · DISCONTINUOUS MEASUREMENT

Momentary Time Sampling

Core definition. The observation period is divided into intervals. The behavior is scored only if it is occurring at the exact moment an interval ends.



Dot = the moment · coral = behavior occurring

4 of 6 moments scored + = 67%

2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Moment

The instant the interval ends, which is the only time the learner is observed.

Scoring

+ if the behavior is occurring at that instant; − if it is not.

Percentage of moments

Moments scored + ÷ total moments × 100.

Discriminative nuances

MTS vs. Partial Interval

MTS ignores what happened earlier in the interval. Partial scores any occurrence during it.

MTS vs. Other Interval Methods

MTS does not require continuous observation, so the RBT can keep teaching between moments.

3 STEP-BY-STEP PROCEDURE

1. **Set the moment schedule**
   Set a timer to cue the end of each interval (e.g., every 60 seconds).
2. **Continue teaching**
   You do not need to watch continuously between cues.
3. **Look at the cue**
   When the timer sounds, observe the learner at that instant.
4. **Score and calculate**
   Mark + or − for that moment only. Calculate percentage at the end.

RBT SCOPE BOUNDARY

RBTs are responsible for implementing MTS and recording each moment accurately. The BCBA chooses the method and the interval length.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

A timer cues every 60 seconds across 6 minutes (6 moments). At each cue the RBT checks whether the learner is in seat. The learner is in seat at 4 of the moments.

4 of 6 moments = 67%

NON-EXAMPLE **Common pitfall**

Incorrect: Scoring interval 2 + because the learner was in seat for the first 20 seconds, then left before the cue.

Why it's wrong: MTS scores only the instant the interval ends. The learner was not in seat at that moment, so interval 2 is −.

5 · EXAM TIP
High-yield summary

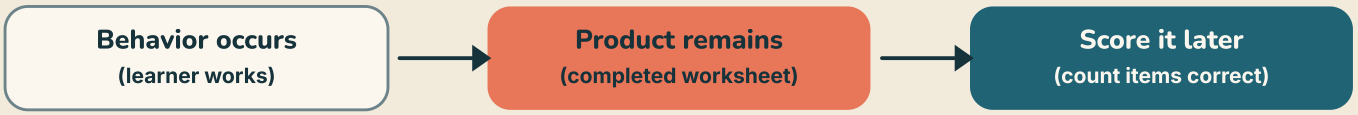
Test takeaway: "At the moment," "at the end of each interval," or "observed only when the timer sounds" points to MTS.

TASK
A.3

1 TASK A.3 · PERMANENT PRODUCT RECORDING

Permanent Product Recording

Core definition. Measuring the lasting products (outcomes) of a behavior after the behavior has occurred, instead of observing the behavior as it happens.



No need to watch the behavior happen; the product can be scored after the session.

2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Permanent product
A tangible, lasting result of a behavior (e.g., a completed worksheet, an assembled puzzle).

Product-based definition
The target is defined by what remains, such as "items answered correctly."

Delayed measurement
Data can be collected after the session because the product stays available.

Discriminative nuances

Permanent Product vs. Direct Observation
Permanent product does not show who made the product or when. Direct observation records the behavior as it occurs.

Product vs. Behavior
Use it only when the product is reliably and only produced by the target behavior.

3 STEP-BY-STEP PROCEDURE

1. **Define the product**
   State exactly what counts (e.g., "items answered correctly").
2. **Save and label the product**
   Keep it and write the learner's name and date on it.
3. **Score the product**
   Count or score the items against the definition.
4. **Record and graph**
   Enter the result on the data sheet and update the graph.

RBT SCOPE BOUNDARY

RBTs are responsible for collecting, scoring, and recording the product data. The BCBA decides when permanent product recording is appropriate.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

A learner completes a 20-item matching worksheet during independent work. After the session the RBT counts the correct items without having watched the learner work.

Recorded: 16 of 20 correct (80%)

NON-EXAMPLE **Common pitfall**

Incorrect: Using permanent product recording to measure hitting that leaves no lasting mark.

Why it's wrong: The behavior produces no lasting product, so there is nothing to score afterward. Use direct observation.

5 · EXAM TIP
High-yield summary

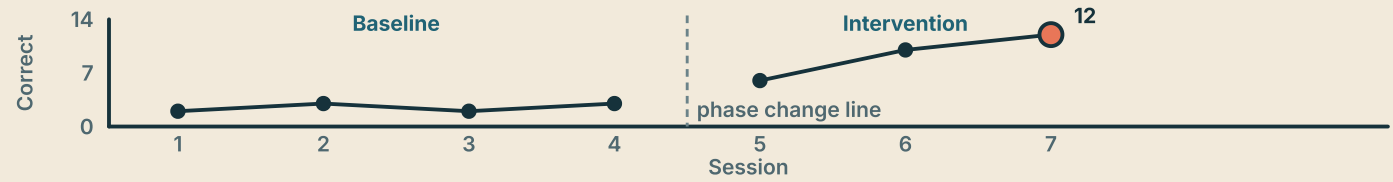
Test takeaway: Look for "after the session," "completed worksheets," or "items assembled." If the observer must see the behavior as it happens, it is not permanent product.

TASK
A.4

1 TASK A.4 · GRAPHING

Entering Data and Updating Graphs

Core definition. Accurately recording each session's data point on the graph so progress can be viewed over time.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Axes
The x-axis shows time (e.g., sessions). The y-axis shows the measure (e.g., number correct).

Data point and data path
Each session is one point. The line connecting consecutive points is the data path.

Phase change line
A vertical line separating conditions, such as baseline and intervention.

Discriminative nuances

Within vs. Across Phases
Connect points within a phase. Never connect points across a phase change line.

Entry Error vs. Real Change
A sudden unexpected jump may be a plotting error. Recheck the data sheet before reporting it.

3 STEP-BY-STEP PROCEDURE

1. **Check the data sheet**
   Confirm the session, the measure, and the units.
2. **Find the position**
   Match the session on the x-axis and the value on the y-axis.
3. **Plot and connect**
   Place one point per session. Connect it to the previous point in the same phase.
4. **Verify and report**
   Recheck the value. Tell the BCBA about anything unusual.

RBT SCOPE BOUNDARY

RBTs are responsible for entering data and updating graphs accurately. The BCBA interprets the graph and decides whether to change programming.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

The data sheet for session 7 shows 12 correct responses. The RBT places a point at session 7, value 12, and connects it to session 6 (value 10) in the intervention phase.

Point plotted: session 7 = 12 correct

NON-EXAMPLE **Common pitfall**

Incorrect: Plotting session 7 at 21 (digits reversed) and drawing the line back to the last baseline point.

Why it's wrong: Reversed digits misstate progress, and a data path must not cross a phase change line.

5 · EXAM TIP
High-yield summary

Test takeaway: Keywords: "plot," "data path," "phase change line." Do not connect data points across a phase change line.

TASK
A.5

1 TASK A.5 · DESCRIBING BEHAVIOR

Observable and Measurable Terms

Core definition. Describing behavior and the environment by what can be seen and counted, without interpretation, so any observer would agree.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Observable
Can be seen or heard by another person.

Measurable
Can be counted, timed, or otherwise quantified.

Interpretation
An inference about feelings or intent (e.g., "angry," "lazy"). Do not use it in data or notes.

Discriminative nuances

Topography vs. Function
Describe what the behavior looks like, not why you think it occurs.

The Stranger Test
If a stranger could not count it from your words alone, the description is not observable and measurable.

3 STEP-BY-STEP PROCEDURE

1. **Watch first**
   Observe what the learner does before writing anything.
2. **Describe the action**
   Use action verbs for what the body does or what the learner says.
3. **Describe the setting**
   State what happened right before and right after, in plain terms.
4. **Run the stranger test**
   Ask whether two observers would count the same thing. Rewrite if not.

RBT SCOPE BOUNDARY

RBTs are responsible for describing behavior and environment objectively in data and session notes. The BCBA writes the operational definitions.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

"The learner drops to the floor and cries for 20 seconds after the RBT says, 'Time to clean up.'"

Observable and measurable: action, setting, duration

NON-EXAMPLE **Common pitfall**

Incorrect: "The learner got angry when asked to clean up."

Why it's wrong: "Angry" is an interpretation. Observers cannot count it, so they cannot agree on it.

5 · EXAM TIP
High-yield summary

Test takeaway: Choose the answer that names actions and conditions, not feelings or intent. "Observable and measurable" rules out words like angry, lazy, or defiant.

TASK
A.6

1 TASK A.6 · CALCULATING AND SUMMARIZING DATA

Rate

Core definition. The number of responses per unit of time: count divided by observation time.

Count		Observation time		Rate
18 responses	÷	45 minutes	=	0.4 per minute

2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Count
The number of responses recorded.

Observation time
The length of the session, in one consistent unit (minutes or hours).

Rate
Count ÷ observation time, reported with its unit (e.g., 0.4 per minute).

Discriminative nuances

Rate vs. Count
Count ignores time. Rate allows fair comparison of sessions of different length.

Rate vs. Percentage
Rate divides by time. Percentage divides by total opportunities or intervals.

3 STEP-BY-STEP PROCEDURE

1. **Record count and time**
   Write the total count and the observation time.
2. **Use one unit of time**
   Convert so the time is in a single unit (e.g., minutes).
3. **Divide count by time**
   Count ÷ time. Never time ÷ count.
4. **Report with the unit**
   Write “per minute” or “per hour” with the answer.

RBT SCOPE BOUNDARY

RBTs are responsible for calculating rate accurately from the data sheet. The BCBA decides which summary to use for decisions.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

A learner makes 18 requests in a 45-minute session.

Rate: 18 ÷ 45 = 0.4 per minute

NON-EXAMPLE **Common pitfall**

Incorrect: Dividing 45 by 18 to get 2.5, or reporting 0.4 with no unit.

Why it’s wrong: Rate is count ÷ time, and it needs its unit. 2.5 is minutes per response, a different number.

5 · EXAM TIP
High-yield summary

Test takeaway: Look for “per minute” or “per hour.” Divide the count by the time and keep the unit.

TASK
A.6

1 TASK A.6 · CALCULATING AND SUMMARIZING DATA

Mean Duration

Core definition. The average length of an episode of behavior: total duration divided by the number of episodes.

Total duration		Number of episodes		Mean duration
2 + 4 + 6 = 12 min	÷	3 episodes	=	4 minutes

2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Total duration
The sum of all episode lengths.

Number of episodes
How many separate times the behavior occurred.

Mean duration
Total duration ÷ number of episodes.

Discriminative nuances

Mean vs. Total Duration
Total is all the time added up. Mean is the average length of one episode.

Mean Duration vs. Rate
Mean duration describes how long each episode lasts. Rate describes how often episodes occur.

3 STEP-BY-STEP PROCEDURE

1. **List each episode**
   Write each episode length from the data sheet.
2. **Add the lengths**
   The sum is the total duration.
3. **Count the episodes**
   Count how many episodes occurred.
4. **Divide**
   Total duration ÷ number of episodes. Include the unit.

RBT SCOPE BOUNDARY

RBTs are responsible for calculating mean duration from the data sheet. The BCBA interprets it and decides what to change.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

A learner has three episodes of out-of-seat behavior lasting 2, 4, and 6 minutes.

Mean duration: 12 ÷ 3 = 4 minutes

NON-EXAMPLE **Common pitfall**

Incorrect: Dividing the 12 total minutes by the 30-minute session length and reporting 0.4.

Why it's wrong: That is a proportion of the session, not mean duration. Divide by the number of episodes (3).

5 · EXAM TIP
High-yield summary

Test takeaway: Look for "average length of each episode." Divide by the number of episodes, not by session time.

TASK
A.6

1 TASK A.6 · CALCULATING AND SUMMARIZING DATA

Percentage

Core definition. A part divided by the whole, multiplied by 100.

Correct trials		Total trials			Percentage
12 correct	÷	15 trials	× 100	=	80%

2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Part (numerator)
The number meeting the criterion, such as correct trials.

Whole (denominator)
The total number of opportunities, such as all trials.

Percentage
Part ÷ whole × 100.

Discriminative nuances

Percent Correct vs. Count Correct
Counts mislead when the number of trials changes. Percentage makes sessions comparable.

Percentage vs. Rate
Percentage divides by opportunities. Rate divides by time.

3 STEP-BY-STEP PROCEDURE

1. **Count the part**
   Count the trials that meet the criterion.
2. **Count the whole**
   Count all trials, correct and incorrect.
3. **Divide part by whole**
   Use a calculator if needed.
4. **Multiply by 100**
   Report with the percent sign.

RBT SCOPE BOUNDARY
RBTs are responsible for calculating percentages accurately from the data sheet. The BCBA decides mastery criteria.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

A learner responds correctly on 12 of 15 trials.

Percentage: 12 ÷ 15 × 100 = 80%

NON-EXAMPLE **Common pitfall**

Incorrect: Dividing correct trials by incorrect trials (12 ÷ 3 = 4) and reporting 400%.

Why it's wrong: The denominator is all opportunities (15). Percent correct cannot exceed 100%.

5 · EXAM TIP
High-yield summary

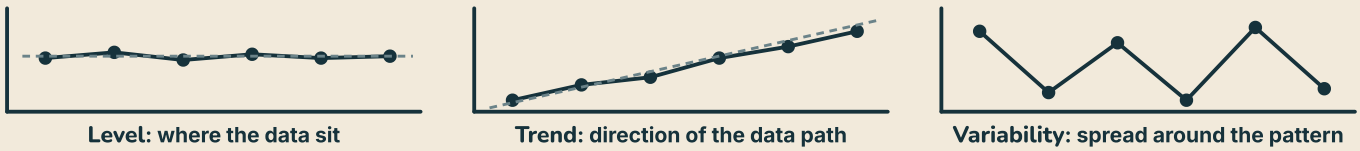
Test takeaway: Look for "percent correct" or "percentage of intervals." The denominator is total opportunities.

TASK
A.7

1 TASK A.7 · INTERPRETING GRAPHS

Identifying Trends in Graphed Data

Core definition. Describing the pattern in a series of data points by its level, trend, and variability.



Level: where the data sit

Trend: direction of the data path

Variability: spread around the pattern

2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Level
Where the data sit on the y-axis (the average value).

Trend
The direction of the data path: increasing, decreasing, or flat.

Variability
How much the data points bounce around the overall pattern.

Discriminative nuances

Trend vs. Level
Data can be high in level and still decreasing. Describe each separately.

Increasing vs. Improving
Increasing is only a direction. For a skill it is improvement. For problem behavior it is a concern.

3 STEP-BY-STEP PROCEDURE

1. **Check axes and phases**
   Read the labels and note where phases change.
2. **Look at level**
   Estimate where the data cluster.
3. **Look at trend and variability**
   Note the direction, and whether points are tight or scattered.
4. **Describe and report**
   State the pattern in plain words and tell the BCBA.

RBT SCOPE BOUNDARY

RBTs are responsible for accurately identifying and describing trends and reporting them. The BCBA analyzes the graph and makes programming decisions.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

Independent requests across six sessions are 2, 3, 3, 5, 6, and 7.

Increasing trend with low variability

NON-EXAMPLE **Common pitfall**

Incorrect: Describing the data 2, 9, 1, 8, 2, 7 as “increasing” because the last point is higher than the first.

Why it’s wrong: The points are highly variable with no consistent direction. Judge the overall pattern, not two points.

5 · EXAM TIP
High-yield summary

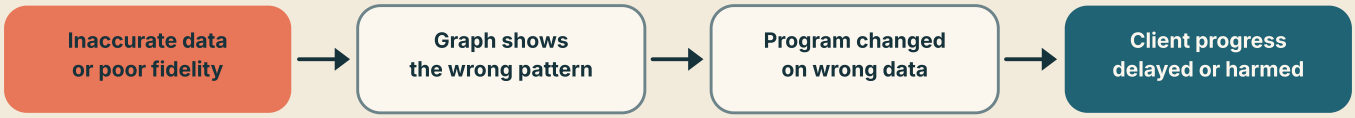
Test takeaway: Keywords: “level, trend, variability.” Trend means direction. Look at the whole data path.

TASK
A.8

1 TASK A.8 · DATA QUALITY

Unreliable Data and Poor Fidelity

Core definition. Describing how inaccurate data collection and incorrect implementation of procedures lead to poor clinical decisions.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Accuracy
How closely recorded data match what actually happened.

Procedural fidelity
How closely a procedure is implemented as written.

Interobserver agreement (IOA)
How closely two observers' data match. Low IOA signals unreliable data.

Discriminative nuances

Data Accuracy vs. Procedural Fidelity
Accuracy is how well you measure. Fidelity is how well you implement.

Different Risks
Poor data hide or invent change. Poor fidelity means the results no longer show whether the written procedure works.

3 STEP-BY-STEP PROCEDURE

1. **Know the definitions and procedures**
   Review them before each session. Ask when anything is unclear.
2. **Record in real time**
   Record as the behavior occurs, not from memory later.
3. **Follow the written protocol**
   Run each step as written. Do not skip or add steps.
4. **Report errors and drift**
   Tell your supervisor about mistakes right away and take part in IOA and fidelity checks.

RBT SCOPE BOUNDARY

RBTs are responsible for collecting data accurately and implementing procedures as written. The BCBA reviews data and fidelity and makes programming decisions.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

An RBT records session data from memory hours later. The estimated counts show a decline, so the BCBA fades the intervention, and the behavior returns.

Risk: a decision made on inaccurate data

NON-EXAMPLE **Common pitfall**

Incorrect: Believing small changes to the protocol do not matter as long as the learner gets correct responses.

Why it's wrong: Changes alter the intervention. The data then no longer show whether the written procedure works.

5 · EXAM TIP
High-yield summary

Test takeaway: Expect items linking inaccurate data or poor fidelity to ineffective or harmful decisions. The answer is to record accurately, follow the protocol, and report problems.

DOMAIN

B

Behavior Assessment

8 QUESTIONS · 11% OF EXAM

3 TASKS · 8 PROFILES

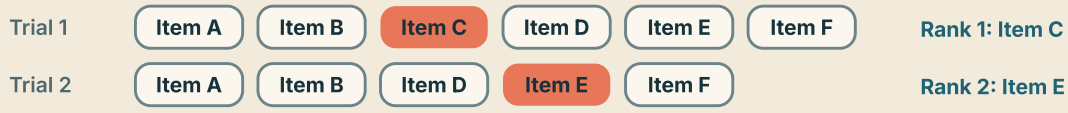
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TASK
B.1

1 TASK B.1 · PREFERENCE ASSESSMENTS

Multiple Stimulus (MSW and MSWO)

Core definition. Several items are presented together and the learner selects among them. Selections are recorded to rank preference.



Chosen item is removed, remaining items are rotated, and the next trial begins.

2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Array
The set of items presented together on a trial, spaced evenly within the learner's reach.

Selection
The first item the learner reaches for or touches within the time allowed by the protocol.

Without replacement (MSWO)
The selected item is removed, the rest are rotated, and the trials continue. The order of selection gives a ranking.

Discriminative nuances

MSWO vs. MSW
MSWO removes each chosen item, so every item gets a rank. MSW (with replacement) returns the chosen item, so it mainly shows the top choices.

Multiple Stimulus vs. Paired Stimulus
Multiple stimulus presents an array. Paired stimulus presents two items at a time.

3 STEP-BY-STEP PROCEDURE

1. **Set up the array**
   Place the items the BCBA selected in a line, evenly spaced, within reach.
2. **Present and wait**
   Say the cue in the protocol (e.g., "Pick one") and wait for a selection.
3. **Record and allow brief access**
   Record the item chosen. Give access to it as the protocol states.
4. **Repeat and rank**
   Remove the item (MSWO), rotate the rest, and repeat until all are chosen. Rank by order chosen.

RBT SCOPE BOUNDARY

RBTs are responsible for conducting the assessment as written and recording each selection. The BCBA chooses the format and items, interprets the results, and decides which items are used as reinforcers.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

An RBT places six items in a line and says, "Pick one." The learner selects Item C first. The RBT removes C, rotates the remaining five, and repeats. The learner selects E next.

Rank 1: Item C. Rank 2: Item E.

NON-EXAMPLE **Common pitfall**

Incorrect: Pointing toward or handing over the item the RBT expects the learner to prefer.

Why it's wrong: Prompting biases the results. The selection must be the learner's own.

5 · EXAM TIP
High-yield summary

Test takeaway: "Array," "without replacement," and "order of selection" point to MSWO. "Two items at a time" points to paired stimulus.

TASK
B.1

1 TASK B.1 · PREFERENCE ASSESSMENTS

Paired Stimulus

Core definition. Items are presented two at a time, in pairs, until every item has been paired with every other item. Selections show which items are chosen most often.



Coral = item selected on that trial. Every item is paired with every other item.

Selection %: A 3 of 3 = 100% · B 2 of 3 = 67% · C 1 of 3 = 33% · D 0 of 3 = 0%

2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Pair
Two items presented together on one trial.

Selection
The item the learner reaches for or touches on a trial.

Selection percentage
Times an item was chosen ÷ times it was presented × 100.

Discriminative nuances

Paired vs. Multiple Stimulus
Paired presents two items per trial and takes more trials. Multiple stimulus presents an array.

Selection Percentage vs. Rank
Rank comes from the percentages: the highest percentage is ranked first.

3 STEP-BY-STEP PROCEDURE

1. **Set up the pairings**
   Pair each item with every other item. Switch which side each item appears on.
2. **Present a pair**
   Say the cue in the protocol and wait for a selection.
3. **Record each choice**
   Record the item chosen. Record no selection if none occurs in the time allowed.
4. **Calculate and rank**
   Work out each item's selection percentage, then rank the items.

RBT SCOPE BOUNDARY
RBTs are responsible for presenting the pairs as written and recording each selection. The BCBA designs the pairings, interprets the percentages, and chooses the reinforcers.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

With four items (A to D) there are six pairs. Item A is chosen on all three trials in which it appears.

Item A: 3 of 3 = 100% (ranked 1st)

NON-EXAMPLE **Common pitfall**

Incorrect: Placing the same item on the learner's right side in every pairing.

Why it's wrong: A side preference can look like an item preference. Counterbalance the sides.

5 · EXAM TIP
High-yield summary

Test takeaway: "Two items at a time" and "selection percentage" point to paired stimulus.

TASK
B.1

1 TASK B.1 · PREFERENCE ASSESSMENTS

Free Operant

Core definition. The learner has free access to a variety of items, and the observer records how long the learner engages with each one.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Free access
All items stay available. The learner is not asked to choose and nothing is removed.

Engagement
Interacting with an item as the BCBA defined it (e.g., touching, manipulating).

Duration of engagement
The time spent engaged with each item. The item with the most time ranks highest.

Discriminative nuances

Free Operant vs. Trial-Based Formats
Free operant has no choice trials. It records how long the learner engages, not which item is picked.

Risk of Problem Behavior
No items are removed, so free operant is less likely to evoke problem behavior than formats that remove items.

3 STEP-BY-STEP PROCEDURE

1. **Set up the environment**
   Place a variety of items within reach.
2. **Allow free access**
   Give no instructions. Let the learner move freely among items.
3. **Time engagement**
   Record how long the learner engages with each item.
4. **Rank by time**
   The item with the longest engagement time ranks first.

RBT SCOPE BOUNDARY

RBTs are responsible for observing and timing engagement as written. The BCBA decides when free operant is used and interprets the results.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

During a 10-minute free-play observation, the learner engages with the trains for 6 minutes, the puzzle for 3 minutes, and the book for 30 seconds.

Highest preference: trains (6 of 10 min)

NON-EXAMPLE **Common pitfall**

Incorrect: Taking an item away after the learner touches it, or telling the learner which item to play with.

Why it's wrong: Free operant means unrestricted access. Removing items or giving instructions changes what the learner does.

5 · EXAM TIP
High-yield summary

Test takeaway: "Free access," "no trials," and "duration of engagement" point to free operant.

TASK
B.2

1 TASK B.2 · SKILL ASSESSMENT

Curriculum-Based Assessment

Core definition. Assessing a learner's skills against the steps of a specific curriculum or skills list to find which skills are strengths and which need teaching.

Skill 1	Skill 2	Skill 3	Skill 4	Skill 5	Skill 6	Skill 7	Skill 8	Skill 9	Skill 10
+	+	–	+	+	+	–	+	–	+

+ = independent (strength) · – = not independent (deficit)

7 strengths · 3 deficits

2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Skills list
The ordered list of skills in the curriculum being assessed.

Strength
A skill the learner performs correctly and independently.

Deficit
A skill the learner does not yet perform independently.

Discriminative nuances

Curriculum-Based vs. Developmental
Curriculum-based compares skills to a teaching curriculum's list. Developmental compares skills to expected milestones for age.

Independent vs. Prompted
Score a skill as a strength only if the learner does it without a prompt.

3 STEP-BY-STEP PROCEDURE

1. **Review the protocol**
   Know the skills list and the scoring rules from the BCBA.
2. **Present each item as written**
   Use the exact instruction. Give no extra help.
3. **Score the response**
   Mark independent correct, incorrect, or prompted as the protocol directs.
4. **Record and submit**
   Complete the data sheet and give it to the BCBA.

RBT SCOPE BOUNDARY

RBTs are responsible for presenting items and scoring them as written. The BCBA chooses the curriculum, interprets the results, and selects teaching targets.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

An RBT presents 10 items from the curriculum's skills list. The learner completes 7 independently.

7 strengths and 3 deficits recorded

NON-EXAMPLE **Common pitfall**

Incorrect: Scoring a response correct after pointing to the answer.

Why it's wrong: A prompted response is not independent, so the skill is scored as a deficit.

5 · EXAM TIP
High-yield summary

Test takeaway: "Assessed against a list of skills" and "strengths and deficits" point to curriculum-based assessment.

TASK
B.2

1 TASK B.2 · SKILL ASSESSMENT

Developmental Assessment

Core definition. Comparing a learner's skills to typical developmental milestones for their age to identify strengths and delays.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Milestone
A skill typically seen within an age range, such as stacking blocks or using two-word phrases.

Delay
A skill that is below the level expected for the learner's age.

Skill area
A group of related skills, such as motor, language, or self-care.

Discriminative nuances

Developmental vs. Curriculum-Based
Developmental shows where skills sit compared with age. Curriculum-based shows which steps of a teaching list are mastered.

Strength vs. Delay
A skill at or above the expected age range is a strength. A skill below it is a delay.

3 STEP-BY-STEP PROCEDURE

1. **Review the checklist**
   Know the skill areas and the scoring rules from the BCBA.
2. **Observe or present tasks**
   Use observation or tasks across skill areas, as the protocol directs.
3. **Score each skill**
   Mark whether the skill is present, as defined on the checklist.
4. **Record and report**
   Record by skill area and give the results to the BCBA.

RBT SCOPE BOUNDARY

RBTs are responsible for observing and scoring as the checklist directs. The BCBA selects and interprets the assessment and chooses the targets.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

Using the BCBA's checklist, an RBT records that during play a learner stacks blocks and uses two-word phrases, both within the age range, but does not use a spoon, which is below it.

Strengths: motor and language. Delay: self-care

NON-EXAMPLE **Common pitfall**

Incorrect: Teaching the checklist items in order, whatever the learner's needs.

Why it's wrong: A developmental checklist shows where skills sit compared with age. The BCBA decides which skills to target.

5 · EXAM TIP
High-yield summary

Test takeaway: "Milestones," "age-expected," and "delay" point to developmental assessment.

TASK
B.2

1 TASK B.2 · SKILL ASSESSMENT

Social Skills Assessment

Core definition. Assessing a learner's social behaviors, such as greetings, turn taking, and conversation, to identify strengths and deficits.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Social skill
An observable behavior used with others, such as greeting back.

Opportunity
A chance to use the skill, such as a peer saying hello.

Response
What the learner does after an opportunity.

Discriminative nuances

Acquisition vs. Performance
A learner may never use a skill, or use it in some settings but not others. Note the setting and people involved.

Opportunities vs. Observations
Count every opportunity, not only the times the learner responded.

3 STEP-BY-STEP PROCEDURE

1. **Review the target skills**
   Know exactly what counts as a correct response.
2. **Observe opportunities**
   Watch in natural settings or arranged opportunities, as the protocol directs.
3. **Record each opportunity**
   Mark each opportunity and whether the learner responded correctly.
4. **Summarize**
   Calculate the percentage of opportunities. Note the setting and people.

RBT SCOPE BOUNDARY
RBTs are responsible for observing and recording opportunities and responses. The BCBA selects the skills, interprets the results, and decides what to teach.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

During recess, five peers greet the learner. The learner greets back after two of them.

2 of 5 opportunities = 40%

NON-EXAMPLE **Common pitfall**

Incorrect: Writing "poor social skills" on the data sheet with no opportunities counted.

Why it's wrong: That is not observable or measurable. Record each opportunity and response.

5 · EXAM TIP
High-yield summary

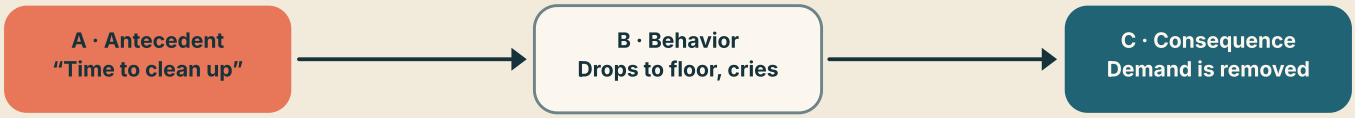
Test takeaway: "Opportunity" and "percentage of opportunities" point to social skills assessment data.

TASK
B.3

1 TASK B.3 · FUNCTIONAL ASSESSMENT

Descriptive Assessment (ABC Data)

Core definition. Direct observation of behavior in the natural environment, recording what happens before, during, and after it, without changing anything.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Antecedent
What happens immediately before the behavior.

Behavior
What the learner does, described in observable terms.

Consequence
What happens immediately after the behavior.

Discriminative nuances

Descriptive vs. Functional Analysis
Descriptive assessment observes natural events. Functional analysis manipulates antecedents and consequences to test a function.

Pattern vs. Proof
ABC patterns suggest a function but do not prove one. The BCBA interprets them.

3 STEP-BY-STEP PROCEDURE

1. **Know the target behavior**
   Use the BCBA's observable definition.
2. **Observe without changing anything**
   Carry on normal routines. Do not alter what you normally do.
3. **Record the A, B, and C**
   Each time the behavior occurs, record the time, what came before, the behavior, and what came after.
4. **Give the data to the BCBA**
   Report exactly what you saw, without guesses about why.

RBT SCOPE BOUNDARY
RBTs participate by collecting ABC data as directed. The BCBA analyzes the patterns and determines the likely function.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

The RBT says, "Time to clean up" (A). The learner drops to the floor and cries (B). The RBT stops the clean-up demand (C).

A: clean-up demand · B: drops, cries · C: demand removed

NON-EXAMPLE **Common pitfall**

Incorrect: Writing, "He was frustrated, so he cried to escape clean-up."

Why it's wrong: That adds an interpretation and a guess about function. Record only what was observed.

5 · EXAM TIP
High-yield summary

Test takeaway: "Antecedent, behavior, consequence" and "natural environment, no manipulation" point to descriptive assessment.

TASK
B.3

1 TASK B.3 · FUNCTIONAL ASSESSMENT

Functional Analysis

Core definition. A systematic assessment that manipulates antecedents and consequences in test conditions and compares behavior with a control condition to identify its function.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Test condition
A condition that arranges one antecedent and one consequence (e.g., attention, demand, alone).

Control condition
A play condition with free access to items and attention and no demands, used for comparison.

Function
The reason the behavior occurs, shown by the condition with the most behavior.

Discriminative nuances

Functional Analysis vs. Descriptive Assessment
Functional analysis manipulates conditions. Descriptive assessment only observes.

Analysis vs. Intervention
The analysis identifies function. Treatment is a separate step designed by the BCBA.

3 STEP-BY-STEP PROCEDURE

1. **Review protocol and safety**
   Review each condition and the safety rules with the BCBA first.
2. **Run each condition as written**
   Arrange the antecedent. Deliver the consequence only as the protocol states.
3. **Collect data throughout**
   Record the behavior in every condition.
4. **Follow the stop criteria**
   End the session as the safety plan directs and report to the BCBA.

RBT SCOPE BOUNDARY
RBTs participate under direct BCBA supervision by running conditions and collecting data as directed. The BCBA designs the conditions, ensures safety, and interprets the graph.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

In the attention condition, the RBT gives a brief reprimand each time the behavior occurs. The behavior occurs 10 times. In the play condition it occurs once.

Most behavior in the attention condition

NON-EXAMPLE **Common pitfall**

Incorrect: Changing how the consequence is delivered partway through a condition.

Why it's wrong: The conditions must stay the same to be compared. A change makes the results unreliable.

5 · EXAM TIP
High-yield summary

Test takeaway: "Manipulates antecedents and consequences" and "control condition" point to functional analysis.

DOMAIN

C

Behavior Acquisition

19 QUESTIONS · 25% OF EXAM

11 TASKS · 21 PROFILES

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TASK
C.1

1 TASK C.1 · REINFORCEMENT

Positive Reinforcement

Core definition. A stimulus is added immediately after a behavior, and the behavior becomes more likely to occur in the future.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Reinforcer
A stimulus that follows a behavior and increases it. It is defined by its effect, not by what it looks like.

Contingency
The reinforcer is delivered only after the target behavior.

Reinforcement effect
An increase in the future occurrence of the behavior.

Discriminative nuances

Positive vs. Negative Reinforcement
Positive adds a stimulus after the behavior. Negative removes or avoids a stimulus. Both increase behavior.

Reinforcer vs. Reward
An item is a reinforcer only if the behavior increases. A reward that does not change behavior is not reinforcement.

3 STEP-BY-STEP PROCEDURE

1. **Identify the reinforcer**
   Use the items on the BCBA's list or the preference assessment results.
2. **Wait for the target behavior**
   Do not deliver the reinforcer before the behavior occurs.
3. **Deliver it right away**
   Deliver immediately after the behavior, with the amount the plan states.
4. **Check the effect**
   Record the behavior across sessions and report if it is not increasing.

RBT SCOPE BOUNDARY
RBTs are responsible for delivering reinforcers as written and recording the behavior. The BCBA selects the reinforcers and decides whether the data show the procedure is working.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

A learner raises their hand. The RBT says, "Great hand raising," and gives a sticker. Across the next sessions hand raising increases.

Hand raising increases: positive reinforcement

NON-EXAMPLE **Common pitfall**

Incorrect: Giving the sticker at the start of the session, before any hand raising.

Why it's wrong: The sticker did not follow the behavior, so it is not contingent. Reinforcement follows the behavior.

5 · EXAM TIP
High-yield summary

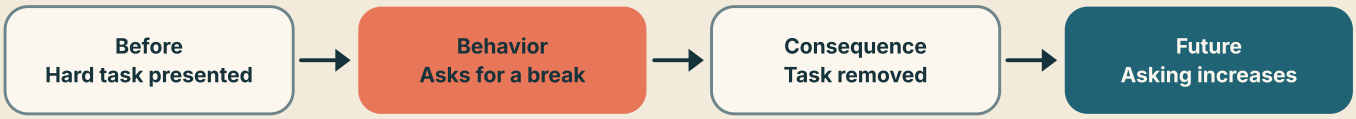
Test takeaway: "Stimulus added" plus "behavior increases" means positive reinforcement. If the behavior does not increase, it was not reinforcement.

TASK
C.1

1 TASK C.1 · REINFORCEMENT

Negative Reinforcement

Core definition. A stimulus is removed or avoided immediately after a behavior, and the behavior becomes more likely to occur in the future.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Aversive stimulus
Something the learner works to get away from or avoid.

Escape
The behavior removes a stimulus that is already present.

Avoidance
The behavior prevents a stimulus from occurring.

Discriminative nuances

Negative Reinforcement vs. Punishment
Negative reinforcement increases the behavior. Punishment decreases it.

"Negative" Means Removed
Negative does not mean bad. It means a stimulus is taken away.

3 STEP-BY-STEP PROCEDURE

1. **Know the aversive stimulus**
   Use the BCBA's plan to know what the learner is escaping or avoiding.
2. **Present the task as written**
   Run the task or condition exactly as the plan states.
3. **Remove it after the target behavior**
   Remove or reduce the stimulus right after the behavior in the plan (e.g., a break request).
4. **Record and report**
   Record whether the behavior is increasing and report what you see.

RBT SCOPE BOUNDARY
RBTs are responsible for implementing the procedure as written and recording the behavior. The BCBA decides which behaviors are reinforced this way and how.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

During a hard task, a learner asks for a break. The RBT ends the task for one minute. Asking for a break increases across sessions.

Break requests increase: negative reinforcement

NON-EXAMPLE **Common pitfall**

Incorrect: Calling it negative reinforcement when the RBT takes away a toy after hitting and the hitting decreases.

Why it's wrong: The behavior decreased, so it is punishment, not reinforcement.

5 · EXAM TIP
High-yield summary

Test takeaway: "Removed," "escape," or "avoid" plus "behavior increases" means negative reinforcement. If the behavior decreases, it is punishment.

TASK
C.1

1 TASK C.1 · REINFORCEMENT

Schedules of Reinforcement

Core definition. A rule that states how many responses, or how much time, must pass before a response is reinforced.

CRF	FR 5	VR 5	FI 2 min	VI 2 min
every response	every 5th response	about every 5th	first after 2 min	first after ~2 min

Ratio schedules count responses. Interval schedules depend on time.

2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Continuous reinforcement (CRF)

Every correct response is reinforced. It is used to teach new skills.

Ratio schedule

Reinforcement follows a number of responses: fixed (FR) is the same number each time, variable (VR) averages around a number.

Interval schedule

Reinforcement follows the first response after time has passed: fixed (FI) is the same time, variable (VI) averages around a time.

Discriminative nuances

Fixed vs. Variable

A fixed requirement stays the same. A variable requirement changes around an average.

Continuous vs. Intermittent

Continuous builds new behavior. Intermittent schedules help keep behavior going with less reinforcement.

3 STEP-BY-STEP PROCEDURE

1. **Know the schedule**
   Read the schedule in the program before the session.
2. **Count responses or time**
   Count responses (ratio) or watch the clock (interval).
3. **Deliver when the rule is met**
   Reinforce only when the requirement is met.
4. **Do not change it yourself**
   Record the data and report to the BCBA, who decides when to thin the schedule.

RBT SCOPE BOUNDARY

RBTs are responsible for implementing the schedule exactly as written. The BCBA selects the schedule and decides when to thin it.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

On an FR 3 schedule, the RBT reinforces every third correct response. The learner gives 9 correct responses.

9 correct responses = 3 reinforcers on FR 3

NON-EXAMPLE **Common pitfall**

Incorrect: Reinforcing after the 2nd, then 5th, then 3rd response on an FR 3 schedule.

Why it's wrong: FR means a fixed number each time. Changing the number each time is a variable schedule.

5 · EXAM TIP
High-yield summary

Test takeaway: "Every nth response" is FR. "On average" is variable. "First response after time passes" is an interval schedule.

TASK
C.1

1 TASK C.1 · REINFORCEMENT

Dimensions of Reinforcement

Core definition. Features of how a reinforcer is delivered that change how well it works: immediacy, contingency, magnitude, intensity, and variety.

Immediacy	Contingency	Magnitude	Intensity	Variety
right after	only after behavior	how much	how strong	rotate items

All five can change how well a reinforcer works.

2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Immediacy
How soon the reinforcer follows the behavior. Sooner is stronger.

Contingency
The reinforcer is delivered only after the behavior.

Magnitude, intensity, variety
Magnitude is the amount, intensity is the strength of the stimulus, and variety is rotating different reinforcers.

Discriminative nuances

Magnitude vs. Intensity
Magnitude is how much (e.g., 2 minutes of play). Intensity is how strong the stimulus is (e.g., enthusiastic praise).

Variety vs. Satiation
Using the same item repeatedly can lead to satiation. Rotating items helps keep it effective.

3 STEP-BY-STEP PROCEDURE

1. **Deliver immediately**
   Give the reinforcer within seconds of the behavior.
2. **Deliver only after the behavior**
   Never give it before or without the behavior.
3. **Match the amount to the plan**
   Use the magnitude and intensity the BCBA states.
4. **Rotate reinforcers**
   Rotate items as the plan states, and report signs the learner is losing interest.

RBT SCOPE BOUNDARY

RBTs are responsible for delivering reinforcers with these dimensions as written. The BCBA sets the dimensions and the list of reinforcers.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

An RBT gives a high-five within two seconds of each correct response and rotates between bubbles, a puzzle, and a song.

Immediate, contingent, and varied

NON-EXAMPLE **Common pitfall**

Incorrect: Delivering the reinforcer five minutes after the response and using the same item all session.

Why it's wrong: A delay weakens the link to the behavior, and one item all session can lead to satiation.

5 · EXAM TIP
High-yield summary

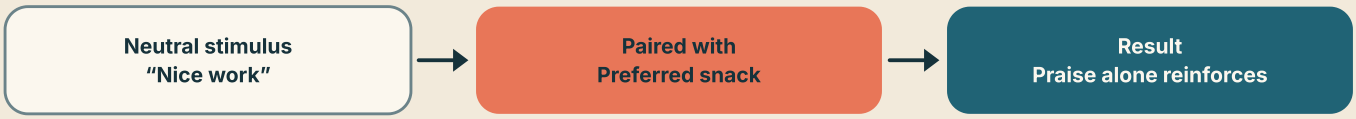
Test takeaway: "Immediately," "only after the behavior," and "rotate items" are the dimensions. "Satiation" points to variety.

TASK
C.2

1 TASK C.2 · CONDITIONED REINFORCERS

Establishing Conditioned Reinforcers

Core definition. A stimulus that was neutral becomes a reinforcer because it has been paired with a stimulus that already works as a reinforcer.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Unconditioned reinforcer
A reinforcer that works without learning, such as food or a drink.

Conditioned reinforcer
A reinforcer that works because of pairing, such as praise or a token.

Pairing
Presenting the neutral stimulus together with, or just before, the existing reinforcer, many times.

Discriminative nuances

Conditioned vs. Unconditioned
Conditioned value is learned through pairing. Unconditioned value does not need to be learned.

Generalized Conditioned Reinforcers
Tokens and money are paired with many reinforcers, so they work across many situations.

3 STEP-BY-STEP PROCEDURE

1. **Know the pair**
   Know the neutral stimulus and the existing reinforcer the BCBA chose.
2. **Pair them**
   Deliver the neutral stimulus (e.g., "Nice work") with or right before the reinforcer.
3. **Repeat across many trials**
   Pair consistently, every time.
4. **Check the effect**
   Record whether behavior increases when the stimulus is delivered alone.

RBT SCOPE BOUNDARY
RBTs are responsible for pairing as written and recording the data. The BCBA selects the stimuli and decides whether the new reinforcer is established.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

An RBT says, "Nice work," and gives a preferred snack across many trials. Later, "Nice work" alone follows correct responses and responding increases.

"Nice work" is now a conditioned reinforcer

NON-EXAMPLE **Common pitfall**

Incorrect: Assuming praise reinforces a learner because it works for other learners.

Why it's wrong: A conditioned reinforcer is learned. It must be established and checked for each learner.

5 · EXAM TIP
High-yield summary

Test takeaway: "Paired with" and "learned value" point to conditioned reinforcers.

TASK
C.3

1 TASK C.3 · DISCRETE-TRIAL TEACHING

Discrete-Trial Teaching (DTT)

Core definition. A teaching format that breaks a skill into small units taught in repeated trials, each with a clear beginning and end.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Discriminative stimulus (S^D)
The instruction or cue that begins the trial.

Response and consequence
The learner's response, followed by reinforcement for a correct response or error correction.

Inter-trial interval (ITI)
The brief pause between one trial and the next.

Discriminative nuances

DTT vs. Natural Environment Training
DTT is instructor-led with a set structure. NET follows the learner's interest in natural settings.

Prompted vs. Independent Trials
Record prompted and independent trials separately so progress is clear.

3 STEP-BY-STEP PROCEDURE

1. **Deliver the S^D**
   Get the learner's attention and give a clear instruction once.
2. **Prompt if needed**
   Use the prompt the plan states, at the time it states.
3. **Deliver the consequence**
   Reinforce a correct response. Follow the error-correction steps for an incorrect one.
4. **Pause and record**
   Pause for the ITI and record the trial before starting the next.

RBT SCOPE BOUNDARY
RBTs are responsible for running trials exactly as written and recording each one. The BCBA writes the targets, prompts, and teaching criteria.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

The RBT says, "Touch red." The learner touches the red card. The RBT says, "Yes!" and gives a token, then pauses three seconds before the next trial.

One trial recorded: independent correct

NON-EXAMPLE **Common pitfall**

Incorrect: Saying, "Touch red, touch red, touch red," before the learner responds.

Why it's wrong: The S^D should be delivered once and clearly. Repeating it blurs where the trial starts.

5 · EXAM TIP
High-yield summary

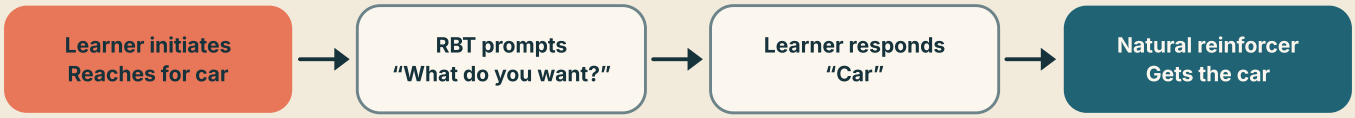
Test takeaway: "Discrete," "clear beginning and end," and the five parts (S^D, prompt, response, consequence, ITI) point to DTT.

TASK
C.4

1 TASK C.4 · NATURALISTIC TEACHING

Incidental Teaching

Core definition. A naturalistic procedure in which the learner starts an interaction and the RBT uses that moment to teach a skill.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Learner initiation
The learner reaches, points, or asks for an item or activity.

Arranged environment
Desired items are placed in view but out of reach to create initiations.

Natural reinforcer
The item or activity the learner asked for.

Discriminative nuances

Incidental Teaching vs. DTT
Incidental teaching starts with the learner. DTT starts with the instructor's instruction.

Incidental Teaching vs. NET
Incidental teaching is one type of naturalistic teaching in which the learner's initiation starts the teaching.

3 STEP-BY-STEP PROCEDURE

1. **Arrange the environment**
   Place preferred items in view and out of reach.
2. **Wait for an initiation**
   Wait for the learner to reach, point, or ask.
3. **Prompt a fuller response**
   Ask for more, as the plan states (e.g., "What do you want?").
4. **Deliver the item and record**
   Give the item after the response and record the trial.

RBT SCOPE BOUNDARY

RBTs are responsible for running incidental teaching as written and recording the data. The BCBA chooses the targets and the level of response required.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

A learner reaches toward a toy car on a shelf. The RBT asks, "What do you want?" The learner says, "Car," and the RBT gives the car.

Natural reinforcer delivered after the response

NON-EXAMPLE **Common pitfall**

Incorrect: Handing over the car the moment the learner reaches, with no response requested.

Why it's wrong: The initiation was not used to teach anything. Prompt a response first.

5 · EXAM TIP
High-yield summary

Test takeaway: "Learner-initiated" and "natural reinforcer" point to incidental teaching.

TASK
C.4

1 TASK C.4 · NATURALISTIC TEACHING

Natural Environment Training (NET)

Core definition. Teaching skills during everyday routines and activities in natural settings, using the learner's interests and natural reinforcers.

Snack time	Play	Getting dressed	Outings
requesting	turn taking	body parts	greetings

Teach inside routines the learner already has, with consequences that naturally follow.

2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Natural environment
The typical settings and routines the learner is in, such as snack time or play.

Natural reinforcer
A consequence that naturally follows the behavior, such as getting the juice that was requested.

Learner interest
The items and activities the learner chooses to engage with.

Discriminative nuances

NET vs. DTT
NET happens within routines and follows the learner's interest. DTT is instructor-led with set trials.

NET vs. Incidental Teaching
NET is the broader group. Incidental teaching is one type that starts with the learner's initiation.

3 STEP-BY-STEP PROCEDURE

1. **Spot the opportunity**
   Find a natural teaching moment in the routine.
2. **Follow the learner's interest**
   Build the opportunity around what the learner wants.
3. **Present a clear opportunity**
   Give the instruction or prompt the plan states.
4. **Deliver the natural reinforcer**
   Give the item or activity and record the response.

RBT SCOPE BOUNDARY

RBTs are responsible for running NET opportunities as written and recording the data. The BCBA chooses the targets and the routines used.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

At snack time the RBT holds the juice and says, "Say juice." The learner says, "Juice," and the RBT pours the juice.

Skill taught in the routine with a natural reinforcer

NON-EXAMPLE **Common pitfall**

Incorrect: Teaching "say juice" at a table with a token as the reinforcer and calling it NET.

Why it's wrong: The setting and the reinforcer are not natural. The juice should be the consequence.

5 · EXAM TIP
High-yield summary

Test takeaway: "Everyday routines" and "natural reinforcer" point to NET.

TASK
C.5

1 TASK C.5 · TASK ANALYSIS CHAINING

Forward Chaining

Core definition. Teaching the steps of a task analysis in order, starting with the first step. The learner performs the step being taught and the RBT completes the rest.



Forward: the learner is taught step 1 first; the RBT completes the rest.

2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Task analysis
A task broken into small steps in order.

Chain
The ordered steps that make up the task.

Mastery criterion
The performance level required before the next step is added.

Discriminative nuances

Forward vs. Backward Chaining
Forward starts with the first step. Backward starts with the last step.

Forward Chaining vs. Total Task
Forward teaches one added step at a time. Total task teaches every step each trial.

3 STEP-BY-STEP PROCEDURE

1. **Review the task analysis**
   Know the steps and the order.
2. **Teach the first step**
   Prompt as the plan states. The RBT completes the remaining steps.
3. **Reinforce the taught step**
   Reinforce as the plan states and record the step.
4. **Add the next step**
   Add the next step only when the mastery criterion is met.

RBT SCOPE BOUNDARY
RBTs are responsible for teaching and recording each step as written. The BCBA writes the task analysis and decides when to add steps.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

In hand washing, the learner turns on the water independently (step 1). The RBT completes steps 2 to 6. Reinforcement follows step 1.

Step 1 taught; step 2 added after mastery

NON-EXAMPLE **Common pitfall**

Incorrect: Starting the teaching at step 4 of 6 because it is the hardest.

Why it's wrong: Forward chaining starts with the first step and moves through the chain in order.

5 · EXAM TIP
High-yield summary

Test takeaway: "Begins with the first step" points to forward chaining.

TASK
C.5

1 TASK C.5 · TASK ANALYSIS CHAINING

Backward Chaining

Core definition. Teaching the steps in reverse order. The RBT completes all steps except the last, which the learner performs. Earlier steps are added backward as each is mastered.



Backward: the RBT completes steps 1 to 5; the learner performs the last step.

2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Last step
The final step of the task, taught first.

Natural reinforcement
Completing the task gives the learner its natural result, such as dry hands.

Backward order
Steps are added from the end of the chain toward the beginning.

Discriminative nuances

Backward vs. Forward Chaining
Backward starts with the last step, so the learner always finishes the task.

Backward Chaining vs. Total Task
Backward teaches one step from the end at a time. Total task teaches every step each trial.

3 STEP-BY-STEP PROCEDURE

1. **Review the task analysis**
   Know the steps and the order.
2. **Complete the earlier steps**
   The RBT completes steps 1 to 5 as written.
3. **Teach the last step**
   Prompt the learner through step 6. Reinforce as the plan states.
4. **Add the next step back**
   Add step 5 only when the mastery criterion is met.

RBT SCOPE BOUNDARY

RBTs are responsible for teaching and recording each step as written. The BCBA writes the task analysis and decides when to add steps.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

In hand washing, the RBT turns on the water, wets the hands, adds soap, scrubs, and rinses. The learner dries their hands (step 6).

Step 6 taught; step 5 added after mastery

NON-EXAMPLE **Common pitfall**

Incorrect: Teaching step 1 first and calling it backward chaining.

Why it's wrong: Backward chaining starts with the last step in the chain.

5 · EXAM TIP
High-yield summary

Test takeaway: "Begins with the last step" and "the learner always completes the task" point to backward chaining.

TASK
C.5

1 TASK C.5 · TASK ANALYSIS CHAINING

Total Task Chaining

Core definition. Teaching every step of the task analysis in each trial, with prompts only on the steps the learner cannot yet do.



Total task: the learner attempts every step each trial; prompts only where needed.

2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Full chain
All steps of the task analysis, in order.

Prompting by step
Prompts are given only on the steps the learner needs help with.

Trial
One pass through every step of the chain.

Discriminative nuances

Total Task vs. Forward or Backward
Total task works on all steps each trial. Forward and backward work on one step at a time.

When It Is Chosen
The BCBA selects total task chaining, often when the learner can already do many of the steps.

3 STEP-BY-STEP PROCEDURE

1. **Review the task analysis**
   Know the steps, the order, and the prompts.
2. **Let the learner start**
   The learner attempts each step in order.
3. **Prompt where needed**
   Prompt only the steps the plan says need help.
4. **Reinforce and record**
   Reinforce at the end and record which steps were independent.

RBT SCOPE BOUNDARY

RBTs are responsible for running each trial as written and recording each step. The BCBA selects the chaining method and writes the prompts.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

A learner attempts all six hand-washing steps. The RBT gives a gestural prompt for the soap step only, then reinforces at the end.

5 of 6 steps independent

NON-EXAMPLE **Common pitfall**

Incorrect: Completing steps 2 to 6 for the learner on every trial.

Why it's wrong: That is forward chaining. In total task chaining the learner attempts every step.

5 · EXAM TIP
High-yield summary

Test takeaway: "All steps in each trial" points to total task chaining.

TASK
C.6

1 TASK C.6 · DISCRIMINATION TRAINING

Discrimination Training

Core definition. Teaching a learner to respond in the presence of one stimulus (the S^D) and not in the presence of another (the SΔ), by reinforcing only the correct response.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

S^D
A stimulus that signals reinforcement is available for the response.

SΔ (S-delta)
A stimulus that signals reinforcement is not available for the response.

Differential reinforcement
Reinforcing the response in the presence of the S^D and not in the presence of the SΔ.

Discriminative nuances

Discrimination vs. Generalization
Discrimination narrows responding to the right stimulus. Generalization widens responding across stimuli.

Stimulus Control
A learner shows stimulus control when they respond reliably to the S^D and not to the SΔ.

3 STEP-BY-STEP PROCEDURE

1. **Set up the S^D and SΔ**
   Place the materials as the plan states (e.g., a red card and a blue card).
2. **Present the instruction**
   Give the instruction once and wait for a response.
3. **Reinforce the correct response**
   Reinforce a response to the S^D right away.
4. **Do not reinforce SΔ responses**
   Follow the error-correction steps and record each trial.

RBT SCOPE BOUNDARY
RBTs are responsible for running trials as written and recording each response. The BCBA selects the targets and writes the error-correction steps.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

The RBT places a red card and a blue card and says, "Touch red." The learner touches red and the RBT reinforces it. Touching blue is not reinforced.

Responding comes under control of the S^D

NON-EXAMPLE **Common pitfall**

Incorrect: Reinforcing the learner for touching blue after they touched red.

Why it's wrong: That reinforces responding to the SΔ, so the discrimination will not develop.

5 · EXAM TIP
High-yield summary

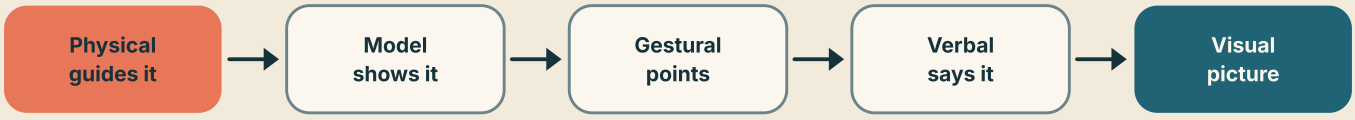
Test takeaway: "S^D," "SΔ," and "stimulus control" point to discrimination training.

TASK
C.7

1 TASK C.7 · PROMPTING AND FADING

Types of Prompts

Core definition. Added cues given before or during a response to help the learner respond correctly. Prompts are later faded.



A typical order from most to least intrusive. The BCBA sets the order.

2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Response prompts
Help given by the RBT: physical guidance, a model, a gesture, or a verbal cue.

Stimulus prompts
Changes to the materials, such as a highlight, a position, or a picture.

Prompt dependence
The learner waits for the prompt instead of responding on their own.

Discriminative nuances

Response vs. Stimulus Prompts
Response prompts change what the RBT does. Stimulus prompts change the materials.

Intrusiveness
More intrusive prompts give more help. A hierarchy orders prompts from most to least intrusive. The BCBA sets the order for each learner.

3 STEP-BY-STEP PROCEDURE

1. **Know the prompt**
   Know the type and the order the plan states.
2. **Deliver it at the planned time**
   Give it after the S^D, or as the plan says.
3. **Reinforce as planned**
   Reinforce prompted responses as the plan states.
4. **Fade it**
   Fade the prompt as the plan states and report any prompt dependence.

RBT SCOPE BOUNDARY

RBTs are responsible for delivering and fading prompts as written and recording the prompt level. The BCBA selects the prompts and the fading plan.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

The RBT says, “Touch your nose,” and touches their own nose. The learner copies and touches their nose.

Model prompt: prompted correct response

NON-EXAMPLE **Common pitfall**

Incorrect: Using the same physical prompt on every trial for weeks.

Why it’s wrong: Prompts must be faded. Otherwise the learner becomes dependent on them.

5 · EXAM TIP
High-yield summary

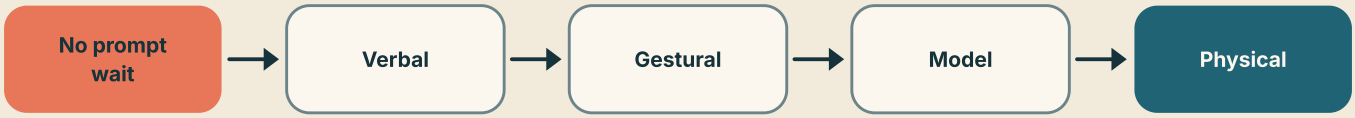
Test takeaway: Know the response prompts (physical, model, gestural, verbal) and the stimulus prompts. “Waits for the prompt” means prompt dependence.

TASK
C.7

1 TASK C.7 · PROMPTING AND FADING

Least-to-Most Prompting

Core definition. A prompt hierarchy that gives the learner a chance to respond independently first, then adds more intrusive prompts only as needed.



No correct response in the time allowed moves to the next prompt.

2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Prompt hierarchy
An ordered list of prompts from least to most intrusive.

Independent opportunity
A chance to respond with no prompt before any help is given.

Escalation
Moving to the next prompt when there is no correct response in the time allowed.

Discriminative nuances

Least-to-Most vs. Most-to-Least
Least-to-most starts with no prompt. Most-to-least starts with the most help.

Errors
Least-to-most lets the learner show independence but allows more errors than errorless methods.

3 STEP-BY-STEP PROCEDURE

1. **Know the hierarchy**
   Know each level and the order.
2. **Deliver the S^D and wait**
   Wait the time the plan states for an independent response.
3. **Add the next prompt**
   If there is no correct response, give the next level.
4. **Reinforce and record**
   Reinforce a correct response and record the prompt level used.

RBT SCOPE BOUNDARY
RBTs are responsible for running the hierarchy as written and recording the prompt level. The BCBA selects the hierarchy and decides when prompts change.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

The RBT says, “Clap your hands,” and waits five seconds. There is no response. A verbal prompt, then a model prompt, follow. The learner claps after the model.

Correct response at the model prompt level

NON-EXAMPLE **Common pitfall**

Incorrect: Starting with a physical prompt on the first trial.

Why it’s wrong: That starts with the most intrusive prompt, which is most-to-least prompting.

5 · EXAM TIP
High-yield summary

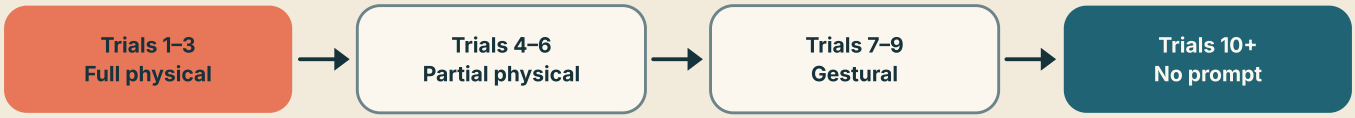
Test takeaway: “Independent opportunity first” and “more prompts only as needed” point to least-to-most prompting.

TASK
C.7

1 TASK C.7 · PROMPTING AND FADING

Errorless Teaching

Core definition. Giving enough prompting at the start for the learner to respond correctly on nearly every trial, then fading the prompt across trials.



The prompt is faded after the learner succeeds at each step.

2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Most-to-least prompting
Start with the most help the learner needs and fade it as they succeed.

Error prevention
The prompt is given before an error can occur.

Prompt fading
Reducing how intrusive the prompt is as the learner succeeds.

Discriminative nuances

Errorless vs. Least-to-Most
Errorless prompts first to prevent errors. Least-to-most waits for an independent response first.

Why It Is Used
Fewer errors can mean less frustration and fewer practiced mistakes.

3 STEP-BY-STEP PROCEDURE

1. **Know the starting prompt**
   Know the first prompt and the fading criteria.
2. **Prompt with the S^D**
   Deliver the prompt right away so the learner cannot err.
3. **Reinforce correct responses**
   Reinforce each correct response as the plan states.
4. **Fade when the criterion is met**
   Move to the less intrusive prompt only when the criterion is met.

RBT SCOPE BOUNDARY

RBTs are responsible for prompting and fading as written and recording the prompt level. The BCBA sets the starting prompt and the fading criteria.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

For “Touch nose,” the RBT gives full physical guidance for three trials, then partial physical, then a gesture, then no prompt.

Prompt faded across trials

NON-EXAMPLE **Common pitfall**

Incorrect: Waiting to see whether the learner makes an error before prompting.

Why it’s wrong: Errorless teaching prompts first to prevent errors.

5 · EXAM TIP
High-yield summary

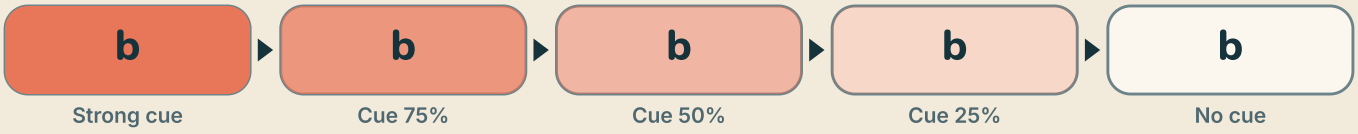
Test takeaway: “Errorless,” “most-to-least,” and “prompt before an error” point to errorless teaching.

TASK
C.7

1 TASK C.7 · PROMPTING AND FADING

Stimulus Fading

Core definition. Gradually changing a stimulus prompt, such as its size, color, or position, until the learner responds correctly to the target stimulus alone.



The cue is faded in small steps until the plain target stimulus is enough.

2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Stimulus prompt
An added feature of the materials that helps the learner respond, such as a highlight.

Target stimulus
The stimulus the learner should respond to without the added prompt.

Fading step
One small change that makes the stimulus prompt less noticeable.

Discriminative nuances

Stimulus Fading vs. Response Prompt Fading
Stimulus fading changes the materials. Response prompt fading changes what the RBT does.

Stimulus Fading vs. Time Delay
Stimulus fading changes the stimulus. Time delay changes how long the RBT waits to prompt.

3 STEP-BY-STEP PROCEDURE

1. **Know the fading steps**
   Know the prompt and each step the plan states.
2. **Present at the current step**
   Use the materials exactly as written for that step.
3. **Reinforce correct responses**
   Reinforce as the plan states.
4. **Move on only at criterion**
   Move to the next step when the criterion is met, and follow the plan after errors.

RBT SCOPE BOUNDARY
RBTs are responsible for preparing materials and fading steps as written and recording the data. The BCBA designs the fading steps and criteria.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

To teach “b” and “d,” the letter b is printed with a bold highlight. Across sessions the highlight is lightened until the learner reads the plain b.

Highlight removed: responds to the plain letter

NON-EXAMPLE **Common pitfall**

Incorrect: Removing the highlight completely after one correct trial.

Why it’s wrong: Stimulus fading is gradual. Remove the prompt in steps, each after the criterion is met.

5 · EXAM TIP
High-yield summary

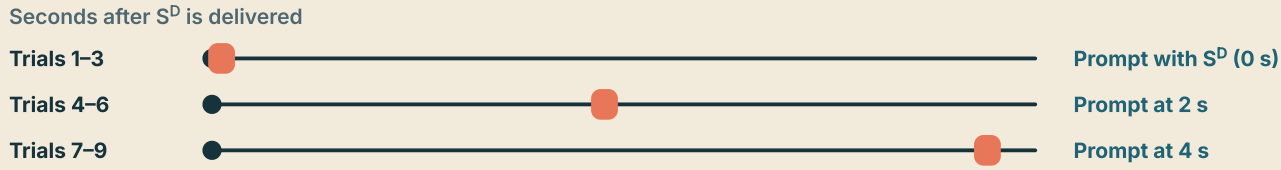
Test takeaway: “Change in the stimulus itself” and “gradually” point to stimulus fading.

TASK
C.7

1 TASK C.7 · PROMPTING AND FADING

Time Delay

Core definition. A prompt-fading procedure that adds a wait between the S^D and the prompt, so the learner can respond before being prompted.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

0-second delay

The prompt is given at the same time as the S^D, so the learner experiences success.

Delay interval

The wait between the S^D and the prompt.

Constant vs. progressive

Constant uses one fixed delay (e.g., 4 seconds). Progressive increases the delay in steps (e.g., 1, 2, 3 seconds).

Discriminative nuances

Time Delay vs. Least-to-Most

Time delay keeps the same prompt and changes only the timing. Least-to-most changes the prompt level.

Anticipation

A correct response before the prompt shows the learner is responding on their own.

3 STEP-BY-STEP PROCEDURE

1. **Start with 0-second trials**
   Deliver the S^D and the prompt together.
2. **Wait the planned delay**
   On later trials, deliver the S^D and wait the set time.
3. **Prompt at the end of the delay**
   If there is no response, give the prompt and reinforce.
4. **Record and increase**
   Record whether the response came before or after the prompt. Increase the delay as the plan states.

RBT SCOPE BOUNDARY

RBTs are responsible for timing the delay and prompting as written and recording the data. The BCBA chooses constant or progressive delay and sets the delay times.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

The RBT shows a cup and asks, “What is this?” At first the RBT says “cup” with the question. Later the RBT waits three seconds, and the learner says “cup” at two seconds, before any prompt.

Correct response before the prompt

NON-EXAMPLE **Common pitfall**

Incorrect: Waiting three seconds on the very first trial before giving any prompt.

Why it’s wrong: Time delay starts with 0-second trials so the learner first experiences successful, reinforced responses.

5 · EXAM TIP
High-yield summary

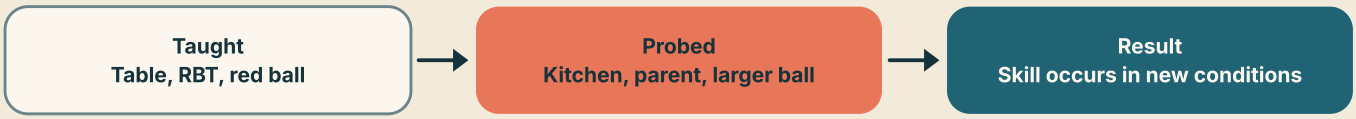
Test takeaway: “Wait between the S^D and the prompt” and “0-second delay” point to time delay.

TASK
C.8

1 TASK C.8 · GENERALIZATION

Generalization

Core definition. A learner performs a skill in settings, with people, and with materials other than those used in teaching.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Stimulus generalization
The skill occurs with new settings, people, or materials.

Response generalization
Different forms of the response that serve the same purpose also occur.

Programming for generalization
Teaching with varied settings, people, and materials, and in natural settings.

Discriminative nuances

Generalization vs. Maintenance
Generalization is performing across conditions. Maintenance is continuing to perform over time.

Generalization vs. Discrimination
Generalization widens responding across stimuli. Discrimination narrows it to the correct stimulus.

3 STEP-BY-STEP PROCEDURE

1. **Know the targets**
   Know which settings, people, and materials the plan targets.
2. **Vary the conditions**
   Teach or probe with different people, settings, and materials, as written.
3. **Use varied instructions and items**
   Use the examples and wording the plan states.
4. **Record by condition**
   Record performance in each condition and report gaps.

RBT SCOPE BOUNDARY
RBTs are responsible for running and recording teaching across conditions as written. The BCBA plans which conditions are targeted and decides when a skill has generalized.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

A learner names a ball with the RBT at the table. A parent asks in the kitchen with a larger ball, and the learner says, “Ball.”

Skill generalized across setting, person, and item

NON-EXAMPLE **Common pitfall**

Incorrect: Counting a skill as generalized because it was correct in the same room with the same RBT and the same card.

Why it’s wrong: Nothing changed, so generalization was not tested. Vary the setting, person, or materials.

5 · EXAM TIP
High-yield summary

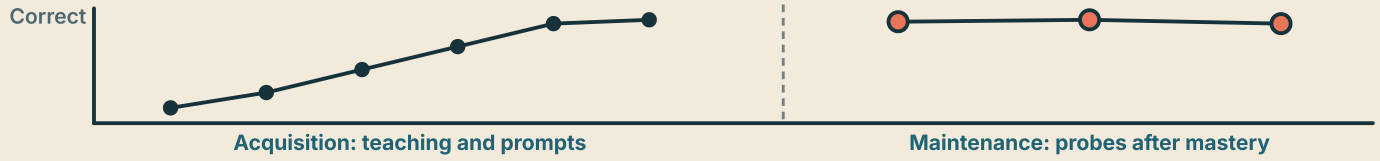
Test takeaway: “Across settings, people, and stimuli” points to generalization.

TASK
C.9

1 TASK C.9 · MAINTENANCE VS. ACQUISITION

Maintenance vs. Acquisition

Core definition. Acquisition is learning a new skill. Maintenance is continuing to perform a mastered skill correctly after teaching has stopped.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Acquisition
The phase in which a new skill is being taught, usually with prompts and frequent reinforcement.

Maintenance
The phase in which a mastered skill is checked over time.

Probe
A check of the skill without teaching or prompts.

Discriminative nuances

Different Procedures
Acquisition uses prompts and frequent reinforcement. Maintenance uses probes and thinner reinforcement.

Maintenance vs. Generalization
Maintenance is performance over time. Generalization is performance across settings, people, and materials.

3 STEP-BY-STEP PROCEDURE

1. **Know each target's phase**
   Check the program to see whether a target is acquisition or maintenance.
2. **Teach acquisition targets**
   Use the prompts and reinforcement the plan states.
3. **Probe maintenance targets**
   Present without prompts and reinforce as the plan states.
4. **Record separately**
   Record each phase separately and report any drop in a maintenance skill.

RBT SCOPE BOUNDARY
RBTs are responsible for running each phase as written and reporting drops in performance. The BCBA decides when a skill is mastered and how maintenance is checked.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

A learner mastered hand washing last month. Each week the RBT presents the instruction without prompts and the learner completes all steps.

Maintenance: the skill is retained

NON-EXAMPLE **Common pitfall**

Incorrect: Giving physical prompts on a mastered target that is in maintenance.

Why it's wrong: Prompting makes it acquisition and hides whether the learner still has the skill.

5 · EXAM TIP
High-yield summary

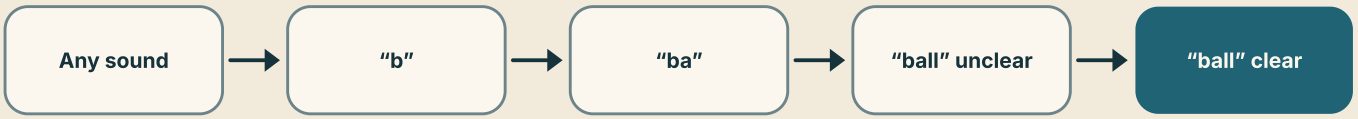
Test takeaway: "Previously mastered" and "after teaching stops" mean maintenance. "New skill" means acquisition.

TASK
C.10

1 TASK C.10 · SHAPING

Shaping

Core definition. Reinforcing successive approximations of a target behavior, while no longer reinforcing the earlier approximations.



Each step is reinforced until steady. Then only the next closer step is reinforced.

2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Target behavior
The final form of the behavior to be taught.

Successive approximation
A response that is closer to the target than the one before it.

Differential reinforcement
Reinforcing closer approximations and no longer reinforcing earlier ones.

Discriminative nuances

Shaping vs. Prompting
Shaping reinforces closer forms the learner already makes. Prompting adds help to get the response.

Shaping vs. Chaining
Shaping changes the form of one response. Chaining links separate steps.

3 STEP-BY-STEP PROCEDURE

1. **Know the target and approximations**
   Know the target and the list of approximations.
2. **Reinforce the current approximation**
   Reinforce responses that meet the current level.
3. **Reinforce only closer forms**
   When the current level is steady, reinforce only the next closer one.
4. **Record the current approximation**
   Record the level reached each session.

RBT SCOPE BOUNDARY

RBTs are responsible for shaping as written and recording the approximations. The BCBA defines the target and approximations and decides when to move up.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

The target is saying "ball." The RBT reinforces any sound, then "b," then "ba," then "ball" said unclearly, then "ball" said clearly.

Closer approximations reinforced; earlier ones no longer

NON-EXAMPLE **Common pitfall**

Incorrect: Continuing to reinforce "ba" after the learner says "ball" clearly.

Why it's wrong: Reinforce only responses that are closer to the target than before.

5 · EXAM TIP
High-yield summary

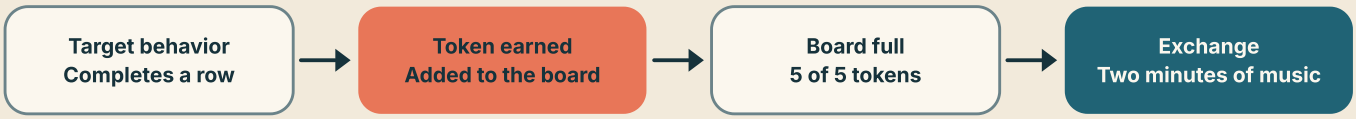
Test takeaway: "Successive approximations" points to shaping.

TASK
C.11

1 TASK C.11 · TOKEN ECONOMIES

Token Economies

Core definition. A system in which a learner earns tokens for target behaviors and exchanges them for backup reinforcers.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Token
An item, such as a sticker or a check mark, earned for a target behavior.

Backup reinforcer
The item or activity the learner trades tokens for.

Exchange rule
How many tokens are needed, and when they can be traded.

Discriminative nuances

Tokens vs. Backup Reinforcers
Tokens have little value alone. They work because they are paired with backups.

Earning vs. Losing
Tokens are earned. Taking tokens away is a different procedure, response cost, and only used if the plan states it.

3 STEP-BY-STEP PROCEDURE

1. **Know the system**
   Know the target behaviors, the token value, and the exchange rule.
2. **Deliver the token right away**
   Give it immediately after the target behavior with brief praise.
3. **Show the tokens**
   Place the token on the board so the learner can see progress.
4. **Exchange as planned**
   Exchange for the backup reinforcer when the rule is met, and record.

RBT SCOPE BOUNDARY
RBTs are responsible for running the token system as written and recording the data. The BCBA designs the system and sets the exchange rules.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

A learner earns a token for each completed worksheet row. At five tokens the learner chooses two minutes of music.

5 tokens exchanged for a backup reinforcer

NON-EXAMPLE **Common pitfall**

Incorrect: Telling the learner, “You lost a token,” for an error when the plan has no response cost.

Why it’s wrong: Removing tokens is a different procedure. Do not use it unless the plan states it.

5 · EXAM TIP
High-yield summary

Test takeaway: “Tokens exchanged for backup reinforcers” points to a token economy.

DOMAIN

D

Behavior Reduction

14 QUESTIONS · 19% OF EXAM

7 TASKS · 21 PROFILES

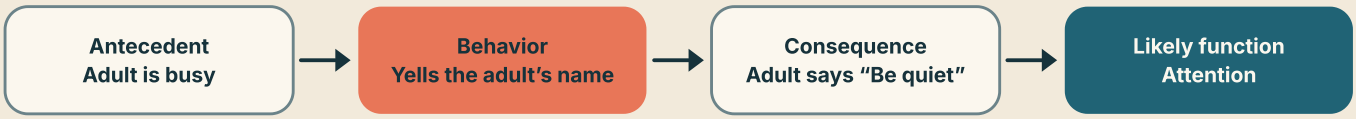
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TASK
D.1

1 TASK D.1 · FUNCTIONS OF BEHAVIOR

Attention

Core definition. Behavior that occurs because it results in attention from others, such as looks, words, or reprimands.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Attention
Any social response from another person, including smiling, talking, or scolding.

Antecedent pattern
The behavior often occurs when attention is low, such as when an adult is busy.

Consequence pattern
Another person attends to the learner right after the behavior.

Discriminative nuances

Attention vs. Tangible
Attention is a social response. Tangible is access to an item or activity.

Reprimands Count
A reprimand is still attention. It can reinforce the behavior it follows.

3 STEP-BY-STEP PROCEDURE

1. **Record the antecedent**
   Note what was happening just before, such as whether anyone was attending.
2. **Record the behavior**
   Describe it in observable terms.
3. **Record the consequence**
   Note whether anyone attended and how.
4. **Report the pattern**
   Give your data to the BCBA, who determines the function.

RBT SCOPE BOUNDARY
RBTs are responsible for recording ABC data and describing patterns objectively. The BCBA determines the function and writes the plan.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

An adult is working at a desk. The learner yells the adult's name. The adult turns and says, "Please be quiet." The pattern repeats.

Likely function: attention

NON-EXAMPLE **Common pitfall**

Incorrect: Ruling out attention because the adult's response was a reprimand.

Why it's wrong: Reprimands are attention, and attention can reinforce behavior.

5 · EXAM TIP
High-yield summary

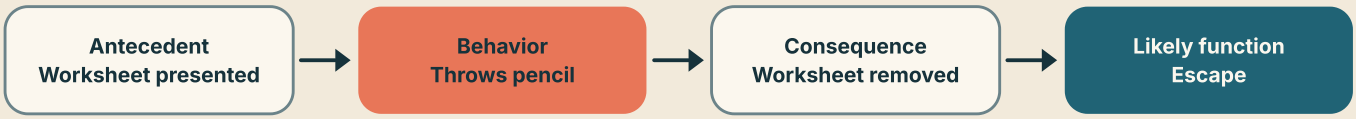
Test takeaway: "Adult or peer attention follows" and "occurs when attention is unavailable" point to an attention function.

TASK
D.1

1 TASK D.1 · FUNCTIONS OF BEHAVIOR

Escape and Avoidance

Core definition. Behavior that occurs because it removes or delays a demand, task, or other unwanted situation.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Escape
The behavior ends a demand or situation that is already present.

Avoidance
The behavior prevents a demand or situation from starting.

Demand
A task, instruction, or request placed on the learner.

Discriminative nuances

Escape vs. Attention
With escape, the demand is removed after the behavior. Attention may follow, but it is not what increases the behavior.

Escape vs. Automatic
Escape-maintained behavior occurs when demands are present. Automatic behavior occurs whether or not demands are present.

3 STEP-BY-STEP PROCEDURE

1. **Record the antecedent**
   Note whether a demand or unwanted situation was present.
2. **Record the behavior**
   Describe it in observable terms.
3. **Record the consequence**
   Note whether the demand was removed, delayed, or changed.
4. **Report the pattern**
   Give your data to the BCBA.

RBT SCOPE BOUNDARY

RBTs are responsible for recording ABC data and describing patterns objectively. The BCBA determines the function and writes the plan.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

The RBT places a worksheet in front of the learner. The learner throws the pencil. The RBT removes the worksheet and gives a break.

Likely function: escape

NON-EXAMPLE **Common pitfall**

Incorrect: Calling it attention because the RBT talked to the learner while removing the task.

Why it's wrong: The function is judged by what increases the behavior. The task was removed.

5 · EXAM TIP
High-yield summary

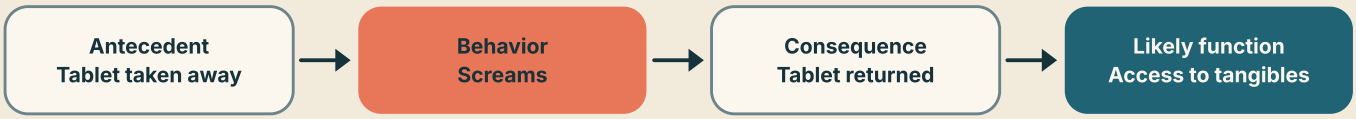
Test takeaway: "Occurs when demands are present" and "ends when the demand is removed" point to an escape function.

TASK
D.1

1 TASK D.1 · FUNCTIONS OF BEHAVIOR

Access to Tangibles

Core definition. Behavior that occurs because it results in access to a preferred item or activity.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Tangible
A preferred item or activity, such as a tablet or a snack.

Access
The learner gets, or gets back, the item or activity after the behavior.

Denied access
The antecedent is often that the item is unavailable or taken away.

Discriminative nuances

Tangible vs. Attention
With tangible, the item or activity is delivered. With attention, a social response is delivered.

Tangible vs. Escape
A learner can get an item by leaving a task. Look at what is delivered and what is removed.

3 STEP-BY-STEP PROCEDURE

1. **Record the antecedent**
   Note whether the item was unavailable or taken away.
2. **Record the behavior**
   Describe it in observable terms.
3. **Record the consequence**
   Note whether the learner got the item or activity.
4. **Report the pattern**
   Give your data to the BCBA.

RBT SCOPE BOUNDARY

RBTs are responsible for recording ABC data and describing patterns objectively. The BCBA determines the function and writes the plan.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

The RBT takes the tablet away at the end of free time. The learner screams. The RBT hands the tablet back.

Likely function: access to tangibles

NON-EXAMPLE **Common pitfall**

Incorrect: Calling it tangible because the learner was holding a toy when the behavior occurred.

Why it's wrong: Function depends on the consequence that increases the behavior, not on which objects are present.

5 · EXAM TIP
High-yield summary

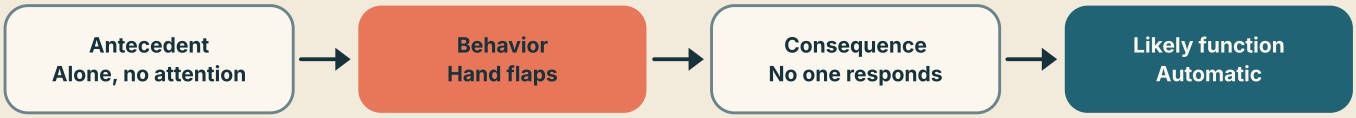
Test takeaway: "Access to a preferred item or activity after the behavior" points to a tangible function.

TASK
D.1

1 TASK D.1 · FUNCTIONS OF BEHAVIOR

Automatic Reinforcement

Core definition. Behavior that occurs because of the sensory or physical effect it produces, whether or not anyone else is present.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Automatic reinforcement
The behavior itself produces the reinforcing effect, such as a sensation.

Sensory stimulation
Input the behavior produces, such as sound or touch.

Across conditions
The behavior occurs alone, during demands, and during free time.

Discriminative nuances

Automatic vs. Social Functions
Social functions depend on what others do. Automatic does not.

Topography vs. Function
The same behavior can serve different functions. Record data across conditions before concluding.

3 STEP-BY-STEP PROCEDURE

1. **Record the conditions**
   Note whether the behavior occurs alone, with demands, and during free time.
2. **Record the behavior**
   Describe it in observable terms.
3. **Record any consequences**
   Note whether anyone responded. Often no one does.
4. **Report the pattern**
   Give your data to the BCBA.

RBT SCOPE BOUNDARY
RBTs are responsible for recording ABC data across conditions and describing patterns objectively. The BCBA determines the function and writes the plan.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

A learner hand flaps while alone in a room, during demands, and during free play. No one responds and the behavior continues.

Likely function: automatic reinforcement

NON-EXAMPLE **Common pitfall**

Incorrect: Assuming hand flapping is automatic without recording data across conditions.

Why it's wrong: The same behavior can have different functions. Data across conditions are needed.

5 · EXAM TIP
High-yield summary

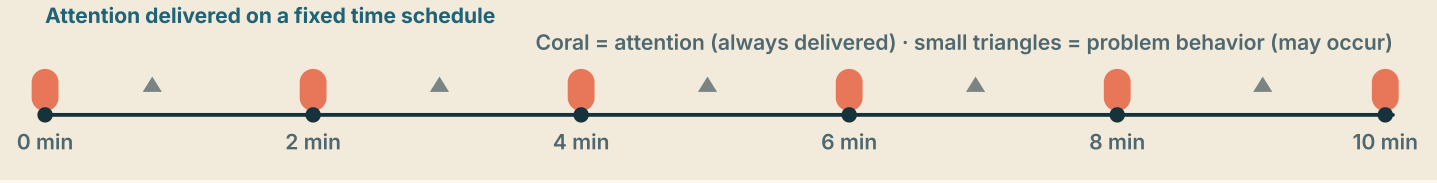
Test takeaway: "Occurs when alone" and "regardless of consequences" point to automatic reinforcement.

TASK
D.2

1 TASK D.2 · ANTECEDENT INTERVENTIONS

Noncontingent Reinforcement (NCR)

Core definition. Delivering a reinforcer on a fixed time schedule, whether or not the problem behavior occurs, to reduce the learner's motivation for the problem behavior.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Noncontingent
Delivery does not depend on the learner's behavior.

Time schedule
A set interval for delivery, such as every 2 minutes.

Matched reinforcer
The reinforcer matches the function, such as attention for attention-maintained behavior.

Discriminative nuances

NCR vs. DRO
NCR delivers on time whatever the behavior. DRO delivers only if the problem behavior has not occurred.

NCR vs. Reinforcement
Reinforcement follows a behavior. NCR follows the clock.

3 STEP-BY-STEP PROCEDURE

1. **Know the reinforcer and schedule**
   Know exactly what to deliver and how often.
2. **Set the timer**
   Start the timer as the plan states.
3. **Deliver on time**
   Deliver at each time point, whether or not the problem behavior occurred.
4. **Record the behavior**
   Record the problem behavior and report changes.

RBT SCOPE BOUNDARY

RBTs are responsible for delivering the reinforcer on schedule as written and recording the behavior. The BCBA selects the reinforcer and schedule and decides when to thin it.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

For attention-maintained yelling, the RBT gives 20 seconds of attention every 2 minutes, whether or not the learner yells.

Attention delivered on time: noncontingent

NON-EXAMPLE **Common pitfall**

Incorrect: Delivering attention only after the learner has been quiet for two minutes.

Why it's wrong: That is DRO. NCR does not depend on the behavior.

5 · EXAM TIP
High-yield summary

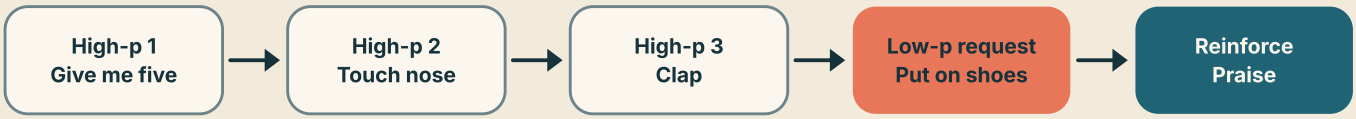
Test takeaway: "Regardless of behavior" and "time-based" point to NCR.

TASK
D.2

1 TASK D.2 · ANTECEDENT INTERVENTIONS

High-Probability Request Sequences

Core definition. Delivering a series of requests the learner usually follows, then a request the learner is less likely to follow, and reinforcing compliance.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

High-probability (high-p) request

A request the learner usually follows, such as "Give me five."

Low-probability (low-p) request

A request the learner is less likely to follow.

Behavioral momentum

Following easy requests builds momentum that carries over to the harder one.

Discriminative nuances

High-p Sequence vs. Demand Fading

A high-p sequence changes the order of requests. Demand fading changes how much is asked over time.

Reinforcement Still Matters

Reinforce compliance with every request, including the low-p request.

3 STEP-BY-STEP PROCEDURE

1. **Know the requests**
   Know which are high-p and which is low-p.
2. **Deliver the high-p requests**
   Deliver two or three quickly and reinforce each.
3. **Deliver the low-p request**
   Deliver it right after the last high-p request.
4. **Reinforce and record**
   Reinforce compliance and record whether the learner complied.

RBT SCOPE BOUNDARY

RBTs are responsible for delivering the sequence as written and recording compliance. The BCBA chooses the requests and decides when to change the sequence.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

The RBT says, "Give me five," "Touch your nose," and "Clap," praising each. Then the RBT says, "Put on your shoes." The learner puts on their shoes.

Compliance with the low-p request, then reinforced

NON-EXAMPLE **Common pitfall**

Incorrect: Pausing several minutes between the last high-p request and the low-p request.

Why it's wrong: The delay loses the momentum. The low-p request should follow right away.

5 · EXAM TIP
High-yield summary

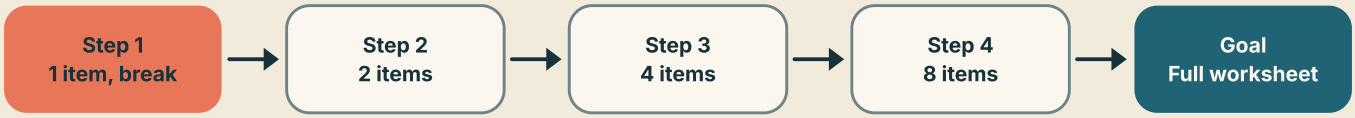
Test takeaway: "Behavioral momentum" and "a series of easy requests first" point to a high-p sequence.

TASK
D.2

1 TASK D.2 · ANTECEDENT INTERVENTIONS

Demand Fading

Core definition. Gradually increasing the number, difficulty, or length of demands over time, starting with an amount the learner handles without problem behavior.



Move up one step only when the criterion is met.

2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Demand
A task, instruction, or request placed on the learner.

Fading step
One small increase in the demand.

Criterion
The performance required before the demand increases.

Discriminative nuances

Demand Fading vs. High-p Sequence

Demand fading changes the amount asked over time. A high-p sequence changes the order of requests.

Demand Fading vs. Escape Extinction

Demand fading increases demands gradually. Escape extinction keeps demands in place after problem behavior.

3 STEP-BY-STEP PROCEDURE

1. **Know the current step**
   Know what amount of demand is in place now.
2. **Present only that amount**
   Present exactly the demands for the current step.
3. **Reinforce compliance**
   Reinforce as the plan states.
4. **Increase only at criterion**
   Move up only when the criterion is met and report if problem behavior returns.

RBT SCOPE BOUNDARY

RBTs are responsible for presenting each step as written and recording the data. The BCBA sets the steps and the criteria for moving up.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

The learner completes 1 worksheet item, then takes a break. After three sessions with no problem behavior, the plan moves to 2 items.

Demand increased from 1 item to 2 items

NON-EXAMPLE **Common pitfall**

Incorrect: Jumping to the full worksheet after one good session.

Why it's wrong: Demands increase in small steps, only after the criterion is met.

5 · EXAM TIP
High-yield summary

Test takeaway: "Gradually increase demands" points to demand fading.

TASK
D.3

1 TASK D.3 · DIFFERENTIAL REINFORCEMENT

DRO (Differential Reinforcement of Other Behavior)

Core definition. Reinforcement is delivered when the problem behavior has not occurred for a set period of time.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

DRO interval
The set period of time with no problem behavior.

Absence of behavior
Reinforcement depends on the problem behavior not occurring during the interval.

Reset
If the plan states it, the interval restarts when the problem behavior occurs.

Discriminative nuances

DRO vs. DRA
DRO reinforces the absence of the problem behavior. DRA reinforces a specific alternative behavior.

DRO vs. NCR
DRO delivers only if the behavior has not occurred. NCR delivers on time whatever the behavior.

3 STEP-BY-STEP PROCEDURE

1. **Know the interval and reinforcer**
   Know the length of the interval and what to deliver.
2. **Start the timer**
   Start the interval as the plan states.
3. **Deliver at the end of a clean interval**
   Deliver the reinforcer if the problem behavior did not occur.
4. **Follow the plan if it occurs**
   Follow the plan (e.g., reset the timer) and record the behavior.

RBT SCOPE BOUNDARY

RBTs are responsible for running the intervals as written and recording the behavior. The BCBA sets the interval and decides when to lengthen it.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

On a 5-minute DRO for hitting, the learner does not hit for 5 minutes. The RBT delivers a sticker.

Interval ended with no hitting: reinforcer delivered

NON-EXAMPLE **Common pitfall**

Incorrect: Delivering the sticker at the end of an interval in which the learner hit.

Why it's wrong: DRO reinforcement requires the absence of the behavior for the whole interval.

5 · EXAM TIP
High-yield summary

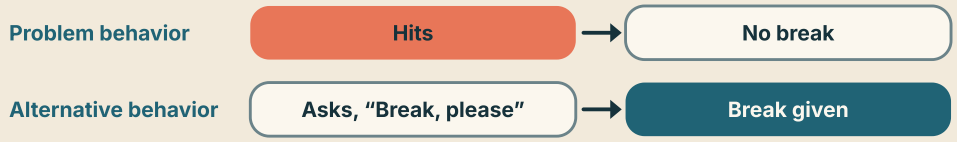
Test takeaway: "Absence of the problem behavior for a time" points to DRO.

TASK
D.3

1 TASK D.3 · DIFFERENTIAL REINFORCEMENT

DRA (Differential Reinforcement of Alternative Behavior)

Core definition. Reinforcing an alternative, appropriate behavior while the problem behavior is no longer reinforced.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Alternative behavior
An appropriate behavior that serves the same function, such as asking for a break.

Problem behavior
The behavior to reduce. It is not reinforced.

Matched reinforcer
The alternative behavior produces the same reinforcer the problem behavior used to produce.

Discriminative nuances

DRA vs. DRI
The alternative in a DRA does not have to be physically incompatible with the problem behavior. In a DRI it is.

DRA vs. FCT
FCT is a DRA in which the alternative behavior is communication.

3 STEP-BY-STEP PROCEDURE

1. **Know the alternative behavior**
   Know what to reinforce and what reinforcer to use.
2. **Reinforce the alternative right away**
   Deliver the reinforcer immediately.
3. **Do not reinforce the problem behavior**
   Follow the plan for what to do when it occurs.
4. **Record both behaviors**
   Record the alternative and the problem behavior.

RBT SCOPE BOUNDARY

RBTs are responsible for reinforcing the alternative behavior as written and recording both behaviors. The BCBA chooses the alternative and writes the plan.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

A learner hits to get a break. The plan teaches asking, "Break, please." The RBT gives a break for asking, and hitting no longer produces one.

Asking is reinforced; hitting is not

NON-EXAMPLE **Common pitfall**

Incorrect: Giving a break for hitting "to calm the learner" as well as for asking.

Why it's wrong: Reinforcing both keeps the problem behavior going.

5 · EXAM TIP
High-yield summary

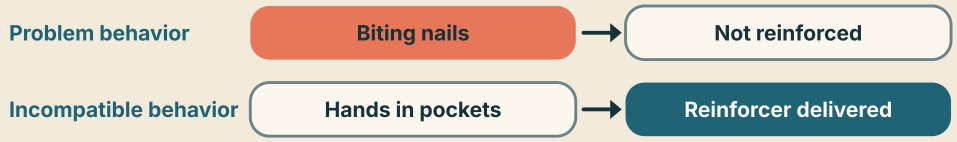
Test takeaway: "Reinforce an appropriate alternative" while the problem behavior is not reinforced points to DRA.

TASK
D.3

1 TASK D.3 · DIFFERENTIAL REINFORCEMENT

DRI (Differential Reinforcement of Incompatible Behavior)

Core definition. Reinforcing a behavior that is physically incompatible with the problem behavior, so the two cannot occur at the same time.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Incompatible behavior
A behavior that cannot occur at the same time as the problem behavior, such as hands in lap and hitting.

Problem behavior
The behavior to reduce.

Reinforcer
Delivered for the incompatible behavior.

Discriminative nuances

DRI vs. DRA
Every DRI is a DRA, but not every DRA is a DRI. DRI requires physical incompatibility.

DRI vs. DRO
DRI reinforces a specific incompatible behavior. DRO reinforces the absence of the problem behavior.

3 STEP-BY-STEP PROCEDURE

1. **Know the incompatible behavior**
   Know exactly what to reinforce.
2. **Watch for it**
   Watch for the incompatible behavior.
3. **Reinforce right away**
   Deliver the reinforcer immediately.
4. **Record both behaviors**
   Record the incompatible and the problem behavior.

RBT SCOPE BOUNDARY

RBTs are responsible for reinforcing the incompatible behavior as written and recording both behaviors. The BCBA selects the behavior and writes the plan.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

For biting nails, the plan reinforces putting hands in pockets. A learner cannot be biting his nails if his hands are in his pockets.

Hands in pockets is reinforced; nail biting cannot occur at the same time

NON-EXAMPLE **Common pitfall**

Incorrect: Reinforcing saying “hello” to reduce hand flapping and calling it DRI.

Why it’s wrong: The learner can say “hello” and flap at the same time. That is not incompatible.

5 · EXAM TIP
High-yield summary

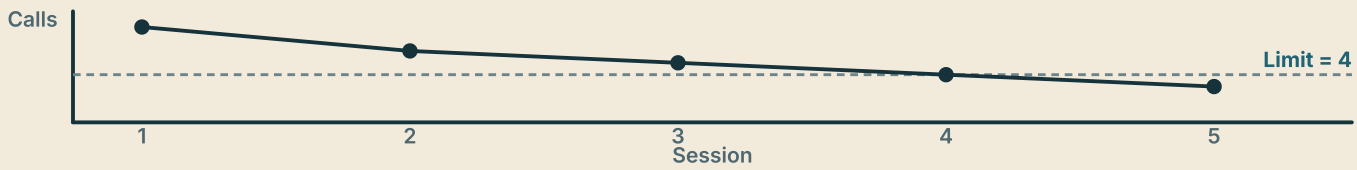
Test takeaway: “Physically incompatible” and “cannot occur at the same time” point to DRI.

TASK
D.3

1 TASK D.3 · DIFFERENTIAL REINFORCEMENT

DRL (Differential Reinforcement of Low Rates)

Core definition. Reinforcing a behavior when it occurs at or below a set limit, to reduce it without eliminating it.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Limit
The most responses allowed while still earning reinforcement.

Full-session DRL
Reinforcement is delivered if the total count for the session is at or below the limit.

Spaced-responding DRL
A response is reinforced only if enough time has passed since the last response.

Discriminative nuances

DRL vs. DRO
DRL allows some behavior. DRO requires none.

When DRL Is Used
It fits behavior that is appropriate at lower levels, such as calling out or asking questions.

3 STEP-BY-STEP PROCEDURE

1. **Know the limit and type**
   Know the limit and whether it is full-session or spaced.
2. **Count the behavior**
   Count or time the behavior as the plan states.
3. **Compare with the limit**
   Check whether the count is at or below the limit.
4. **Reinforce and record**
   Deliver the reinforcer if the limit was met, and record the count.

RBT SCOPE BOUNDARY

RBTs are responsible for counting the behavior and delivering reinforcement as written. The BCBA sets the limit and lowers it over time.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

The limit is 4 or fewer call-outs in a session. The learner calls out 3 times, and the RBT delivers the reinforcer.

3 is at or below the limit of 4: reinforcer delivered

NON-EXAMPLE **Common pitfall**

Incorrect: Withholding reinforcement because the learner called out 3 times.

Why it's wrong: DRL allows behavior at or below the limit. Reinforcement is withheld only above it.

5 · EXAM TIP
High-yield summary

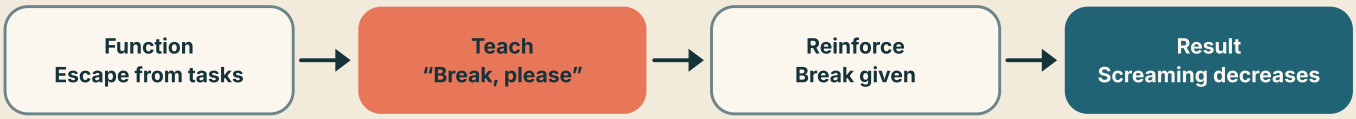
Test takeaway: "Reduce but not eliminate" and "at or below a limit" point to DRL.

TASK
D.3

1 TASK D.3 · DIFFERENTIAL REINFORCEMENT

Functional Communication Training (FCT)

Core definition. Teaching a communication response that serves the same function as the problem behavior and reinforcing it in place of the problem behavior.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Functional communication response (FCR)

The taught communication, such as "Break, please," a card, a sign, or an AAC device.

Matched function

The FCR produces the same reinforcer the problem behavior did.

Problem behavior

No longer produces the reinforcer, as the plan states.

Discriminative nuances

FCT vs. DRA

FCT is a DRA in which the alternative behavior is communication.

Early Teaching

Early on, each FCR is reinforced immediately so it works better than the problem behavior.

3 STEP-BY-STEP PROCEDURE

1. **Know the FCR and function**
   Know which communication to teach and what it should produce.
2. **Prompt the FCR**
   Prompt it when the learner would otherwise engage in the problem behavior, as the plan states.
3. **Reinforce each FCR**
   Deliver the matched reinforcer immediately.
4. **Follow the plan for problem behavior**
   Do not reinforce it. Record both behaviors.

RBT SCOPE BOUNDARY

RBTs are responsible for prompting and reinforcing the FCR as written and recording both behaviors. The BCBA selects the FCR and writes the teaching and thinning plan.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

A learner screams to escape tasks. During hard tasks the RBT prompts, "Break, please," and gives a break. Screaming no longer produces a break.

FCR reinforced with a break

NON-EXAMPLE **Common pitfall**

Incorrect: Teaching the learner to say "help" when the function is attention.

Why it's wrong: The FCR must serve the same function. "Help" will not replace an attention-seeking behavior.

5 · EXAM TIP
High-yield summary

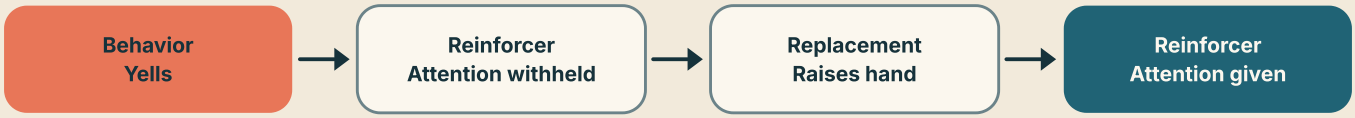
Test takeaway: "Communication that serves the same function" points to FCT.

TASK
D.4

1 TASK D.4 · EXTINCTION

Extinction

Core definition. Withholding the reinforcer that previously maintained a behavior, so the behavior decreases over time.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Reinforcer

The consequence that maintained the behavior. It must be the one identified for that behavior.

Extinction procedure

The reinforcer is no longer delivered after the behavior.

Extinction effect

The behavior decreases after repeated occurrences without the reinforcer.

Discriminative nuances

Extinction vs. Ignoring

Extinction withholds the specific reinforcer that maintains the behavior. Ignoring is extinction only for attention-maintained behavior.

Extinction vs. Punishment

In extinction, nothing new is added or removed. The existing reinforcer stops following the behavior.

3 STEP-BY-STEP PROCEDURE

1. **Know the function**
   Confirm the function in the plan. The reinforcer to withhold depends on it.
2. **Withhold the reinforcer**
   Do not deliver it after the behavior, every time.
3. **Reinforce the replacement**
   Deliver the reinforcer for the replacement behavior as written.
4. **Record the behavior**
   Record each occurrence and the response you gave.

RBT SCOPE BOUNDARY

RBTs are responsible for implementing extinction exactly as written, every time, and recording the behavior. The BCBA selects extinction and writes the plan.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

A learner yells to get attention. The plan says no attention for yelling and attention for a quiet hand raise. The RBT does not respond to yelling and responds to hand raises.

Yelling is no longer reinforced; hand raises are

NON-EXAMPLE **Common pitfall**

Incorrect: Responding to yelling only some of the time.

Why it's wrong: Occasional reinforcement is intermittent reinforcement. It makes the behavior stronger and slower to decrease.

5 · EXAM TIP
High-yield summary

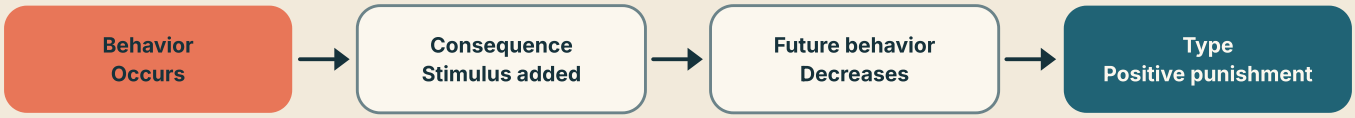
Test takeaway: "Withhold the reinforcer that maintained the behavior" points to extinction.

TASK
D.5

1 TASK D.5 · PUNISHMENT

Positive Punishment

Core definition. Adding a stimulus right after a behavior, which makes the behavior less likely to occur in the future.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Positive
A stimulus is added.

Punishment
Future probability of the behavior decreases.

Punisher
The added stimulus, defined by its effect on the behavior, not by how it looks.

Discriminative nuances

Positive vs. Negative Punishment
Positive punishment adds a stimulus. Negative punishment removes one.

Punishment vs. Reinforcement
Punishment lowers future behavior. Reinforcement raises it. Judge by the effect.

3 STEP-BY-STEP PROCEDURE

1. **Follow the plan**
   Use only procedures written in the plan and approved by the BCBA.
2. **Deliver as written**
   Deliver the consequence exactly as described, right after the behavior.
3. **Record the behavior**
   Record occurrences before and after the procedure.
4. **Report concerns**
   Report any change in the learner or effect on the behavior to the BCBA.

RBT SCOPE BOUNDARY
RBTs are responsible for implementing only procedures written in the plan and reporting concerns. The BCBA selects punishment procedures and writes the plan.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

A young child reaches out and grabs a candy bar from the display shelf in the checkout lane without permission. In response, the mother immediately swats the child's hand away and delivers a sharp verbal reprimand. Over several occurrences, the child reaches for the candy less often.

Stimulus added, behavior decreased: positive punishment

NON-EXAMPLE **Common pitfall**

Incorrect: Calling a consequence punishment because it seems unpleasant, while behavior stays the same or increases.

Why it's wrong: Punishment is defined by a decrease in future behavior, not by how the consequence seems.

5 · EXAM TIP
High-yield summary

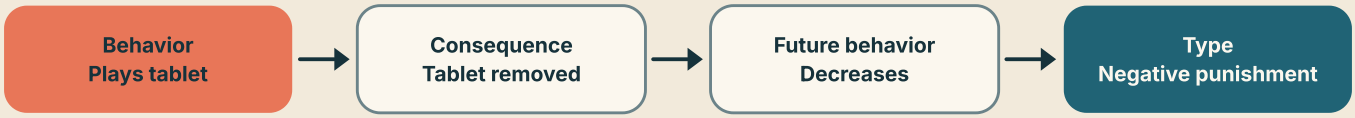
Test takeaway: "Added" plus "behavior decreases" points to positive punishment.

TASK
D.5

1 TASK D.5 · PUNISHMENT

Negative Punishment

Core definition. Removing a stimulus right after a behavior, which makes the behavior less likely to occur in the future.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Negative
A stimulus is removed.

Response cost
Loss of an item or reinforcers after the behavior.

Time-out
Loss of access to reinforcement for a set period after the behavior.

Discriminative nuances

Negative Punishment vs. Negative Reinforcement
Both remove a stimulus. Punishment decreases behavior. Reinforcement increases it.

Negative Punishment vs. Extinction
Negative punishment removes a reinforcer after the behavior. Extinction stops producing it.

3 STEP-BY-STEP PROCEDURE

1. **Follow the plan**
   Use only procedures written in the plan and approved by the BCBA.
2. **Remove as written**
   Remove the item or access exactly as described, right after the behavior.
3. **Record the behavior**
   Record occurrences before and after.
4. **Report concerns**
   Report any change in behavior or the learner to the BCBA.

RBT SCOPE BOUNDARY
RBTs are responsible for implementing only procedures written in the plan and recording the behavior. The BCBA selects the procedure and writes the plan.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

A student decides to play a mobile game on their tablet during a silent reading period, directly violating classroom rules. Because of this behavior, the teacher immediately steps in and confiscates the tablet for the remainder of the day. The next time silent reading is implemented, the student participates in reading.

Stimulus removed, behavior decreased: negative punishment

NON-EXAMPLE **Common pitfall**

Incorrect: Calling it negative punishment when the behavior increases.

Why it's wrong: An increase means the removal acted as negative reinforcement.

5 · EXAM TIP
High-yield summary

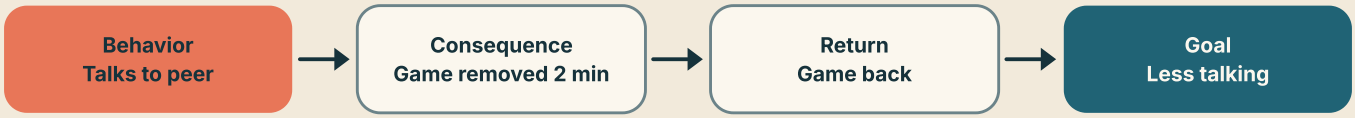
Test takeaway: "Removed" plus "behavior decreases" points to negative punishment.

TASK
D.5

1 TASK D.5 · PUNISHMENT

Time-Out

Core definition. A brief period with no access to reinforcement after a behavior, used to decrease that behavior.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Time-out from reinforcement
Access to reinforcement is removed for a set period.

Non-exclusionary
The learner stays in the area but loses access to reinforcement.

Exclusionary
The learner is moved to a separate area. Use only when the plan allows it.

Discriminative nuances

Time-Out vs. Escape
Time-out must remove a reinforcing situation. If the learner wanted to leave the task, time-out may reinforce the behavior.

Time-Out vs. Extinction
Time-out removes access after the behavior. Extinction withholds the reinforcer the behavior used to produce.

3 STEP-BY-STEP PROCEDURE

1. **Follow the plan**
   Use the exact procedure, length, and location written in the plan.
2. **Start immediately**
   Start right after the behavior, with little talking.
3. **Keep the learner safe**
   Keep the learner safe and supervised the whole time.
4. **Return and record**
   Return the learner to activity as written and record the behavior and length.

RBT SCOPE BOUNDARY
RBTs are responsible for implementing time-out only as written in the plan and keeping the learner safe. The BCBA selects time-out and writes the plan.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

During a game, a learner is talking to a peer when they should be listening to the gym teacher. The RBT removes the game and the learner sits without activity for 2 minutes, as written. Then the game returns.

Loss of access to the game for 2 minutes

NON-EXAMPLE **Common pitfall**

Incorrect: Sending a learner to a quiet area after a behavior maintained by escape from demands.

Why it's wrong: The learner escapes the task, so the behavior can increase.

5 · EXAM TIP
High-yield summary

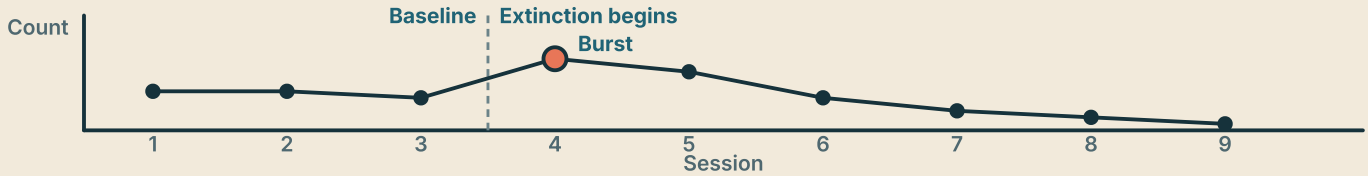
Test takeaway: "Time-out from positive reinforcement" points to negative punishment.

TASK
D.6

1 TASK D.6 · EXTINCTION SIDE EFFECTS

Extinction Burst

Core definition. A temporary increase in the frequency, duration, or intensity of a behavior when extinction first begins.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Burst
The behavior gets worse before it gets better.

Why it happens
The behavior used to produce the reinforcer. The learner tries it harder.

Temporary
The behavior decreases again if extinction is carried out consistently.

Discriminative nuances

Burst vs. Failure
A burst does not mean extinction is failing. Giving in at the burst reinforces the stronger behavior.

Burst vs. Resurgence
A burst occurs when extinction begins. Resurgence occurs later, when a replacement behavior stops working.

3 STEP-BY-STEP PROCEDURE

1. **Expect it**
   Know that extinction can cause a burst before starting.
2. **Stay consistent**
   Keep following the plan. Do not give in.
3. **Keep everyone safe**
   Follow the safety steps in the plan if the behavior becomes unsafe.
4. **Report it**
   Report the increase and record data to the BCBA.

RBT SCOPE BOUNDARY

RBTs are responsible for staying consistent with the plan, keeping everyone safe, and reporting and recording the increase. The BCBA decides whether to change the plan.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

The plan starts extinction for yelling. In session 4, yelling rises from 6 to 11. It decreases to 5, then lower, as the RBT stays consistent.

Temporary increase, then a decrease

NON-EXAMPLE **Common pitfall**

Incorrect: Giving attention at the peak because the plan seems not to be working.

Why it's wrong: This reinforces the stronger behavior and can make it worse later.

5 · EXAM TIP
High-yield summary

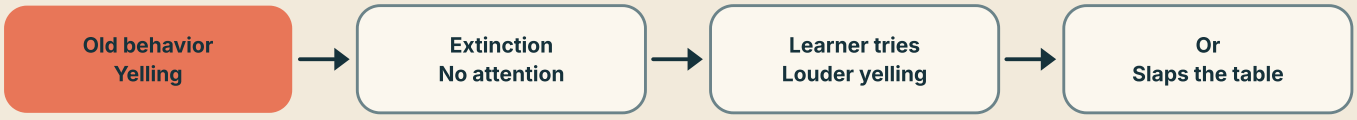
Test takeaway: "Temporary increase when extinction begins" points to an extinction burst.

TASK
D.6

1 TASK D.6 · EXTINCTION SIDE EFFECTS

Response Variation

Core definition. New or different forms of behavior that appear when a previously reinforced behavior is no longer reinforced.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Variation
Different forms of the behavior, such as louder, longer, or a new behavior.

Cause
Extinction. The old behavior stops working, so the learner tries other responses.

Possible outcome
A new behavior may be reinforced by accident.

Discriminative nuances

Variation vs. Burst
A burst is more of the same behavior. Variation is a different form.

Variation vs. Replacement
A replacement behavior is taught. Variation appears without teaching.

3 STEP-BY-STEP PROCEDURE

1. **Watch for new behaviors**
   Watch for new forms of behavior after extinction begins.
2. **Follow the plan**
   Respond to new behaviors as the plan describes. Do not reinforce them by accident.
3. **Record them**
   Describe new behaviors in observable terms.
4. **Report them**
   Report new behaviors to the BCBA.

RBT SCOPE BOUNDARY

RBTs are responsible for watching for, describing, and reporting new behaviors and following the plan. The BCBA updates the plan for new behaviors.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

Yelling no longer gets attention. The learner starts yelling louder, then slapping the table. The RBT records both and tells the BCBA.

New forms of behavior appear under extinction

NON-EXAMPLE **Common pitfall**

Incorrect: Reinforcing the new behavior because it is "better" than the old one.

Why it's wrong: Reinforcing any new behavior can teach the learner to use it instead.

5 · EXAM TIP
High-yield summary

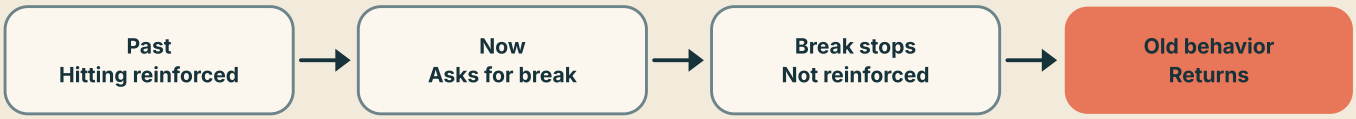
Test takeaway: "New or different forms during extinction" points to response variation.

TASK
D.6

1 TASK D.6 · EXTINCTION SIDE EFFECTS

Resurgence

Core definition. The return of a previously reinforced behavior when a current behavior is no longer reinforced.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Previously reinforced behavior
A behavior that worked in the past, then stopped.

Current behavior
The behavior that now produces the reinforcer, such as a replacement behavior.

Trigger
The current behavior stops producing the reinforcer.

Discriminative nuances

Resurgence vs. Relapse
Resurgence is a return of a past behavior because a current one fails. Relapse is a general return of behavior after treatment.

Resurgence vs. Burst
A burst occurs at the start of extinction. Resurgence occurs later, when a replacement stops working.

3 STEP-BY-STEP PROCEDURE

1. **Keep replacement behavior reinforced**
   Deliver reinforcement for the replacement behavior as written.
2. **Watch for old behaviors**
   Watch for the return of behaviors from the past.
3. **Do not reinforce them**
   Follow the plan for the old behavior.
4. **Report it**
   Report the return and any change in reinforcement to the BCBA.

RBT SCOPE BOUNDARY

RBTs are responsible for reinforcing the replacement behavior as written and reporting any return of old behavior. The BCBA updates the plan.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

A learner used to hit, then learned to ask for a break. Breaks are paused during a staff absence. The learner starts hitting again.

Old behavior returns when the replacement is not reinforced

NON-EXAMPLE **Common pitfall**

Incorrect: Calling it a new behavior because it has not been seen recently.

Why it's wrong: A behavior that was reinforced in the past and returns is resurgence.

5 · EXAM TIP
High-yield summary

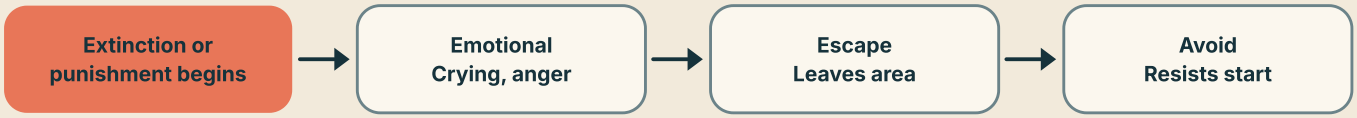
Test takeaway: "Old behavior returns when the replacement stops working" points to resurgence.

TASK
D.6

1 TASK D.6 · EXTINCTION SIDE EFFECTS

Emotional Responding, Escape, and Avoidance

Core definition. Emotional behavior, attempts to leave, or attempts to avoid the situation that can occur as secondary effects of extinction or punishment.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Emotional responding
Crying, anger, or other emotional behavior after reinforcement stops or a punisher is delivered.

Escape
Trying to leave the situation.

Avoidance
Trying to prevent the situation from starting.

Discriminative nuances

Side effect vs. Function
These can follow extinction or punishment. They are separate from the function of the original behavior.

Expected vs. Unsafe
Emotional behavior can be expected. Unsafe behavior must follow the safety steps in the plan.

3 STEP-BY-STEP PROCEDURE

1. **Expect it**
   Know that these can happen when extinction or punishment begins.
2. **Stay consistent**
   Continue the plan. Stay calm and neutral.
3. **Keep safe**
   Follow the safety steps in the plan if needed.
4. **Record and report**
   Record what you see and report it to the BCBA.

RBT SCOPE BOUNDARY

RBTs are responsible for staying calm and consistent, keeping safe, and recording and reporting these responses. The BCBA adjusts the plan.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

After extinction begins, a learner cries, pushes away the materials, and runs toward the door at the start of sessions. The RBT stays calm, follows the plan, and reports it.

Emotional and escape behavior recorded and reported

NON-EXAMPLE **Common pitfall**

Incorrect: Stopping the extinction plan when the learner cries.

Why it's wrong: Stopping reinforces the behavior and the crying. Changes to the plan come from the BCBA.

5 · EXAM TIP
High-yield summary

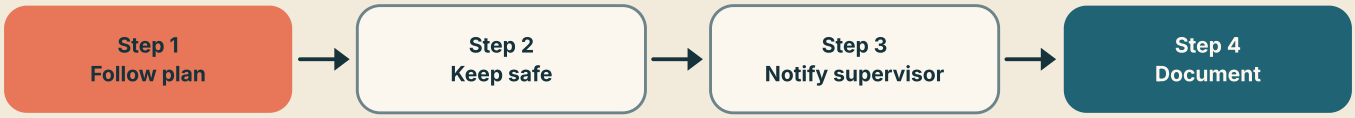
Test takeaway: "Crying, anger, or escape after extinction or punishment" points to secondary effects.

TASK
D.7

1 TASK D.7 · CRISIS AND EMERGENCY PROCEDURES

Crisis and Emergency Procedures

Core definition. Steps taken to keep everyone safe when behavior becomes dangerous, using the written crisis plan.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Crisis plan
The written plan approved by the supervisor for dangerous behavior.

Emergency
A situation with risk of harm that requires immediate steps, which can include medical help.

Incident report
The objective written record of what happened, per the organization's policy.

Discriminative nuances

Crisis vs. Challenging Behavior
Challenging behavior is handled by the behavior plan. A crisis involves danger and follows the crisis plan.

Scope
Use only procedures you were trained on and that the plan allows. Follow the organization's policy.

3 STEP-BY-STEP PROCEDURE

1. **Follow the written plan**
   Follow the crisis plan and the organization's policy.
2. **Keep everyone safe**
   Keep the learner and others safe. Get help if needed.
3. **Notify the supervisor**
   Tell the supervisor right away, as written in the plan.
4. **Document**
   Write an objective account of the event, as policy requires.

RBT SCOPE BOUNDARY

RBTs are responsible for following the written crisis plan, keeping everyone safe, notifying the supervisor, and documenting. The BCBA writes the crisis plan and decides on changes.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

During a session, a learner begins hitting others and does not stop. The RBT follows the crisis plan, keeps others safe, calls the supervisor, and writes an objective report.

Plan followed, supervisor notified, event documented

NON-EXAMPLE **Common pitfall**

Incorrect: Writing in the report that the learner "was angry and wanted attention."

Why it's wrong: Reports describe what happened. Opinions and guesses about why do not belong.

5 · EXAM TIP
High-yield summary

Test takeaway: "Follow the plan, keep safe, notify, document" are the steps. Document objectively.

DOMAIN

E

Documentation and Reporting

10 QUESTIONS · 13% OF EXAM

4 TASKS · 4 PROFILES

E.1 Communicating Team Concerns and Suggestions 75

E.2 Seeking and Prioritizing Clinical Direction 76

E.3 Reporting Variables That Affect Client Progress 77

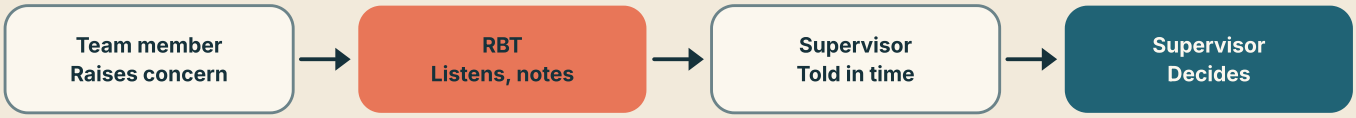
E.4 Communicating Objectively What Occurred 78

TASK
E.1

1 TASK E.1 · COMMUNICATION WITH THE TEAM

Communicating Team Concerns and Suggestions

Core definition. Passing concerns and suggestions from caregivers, teachers, and other providers to the supervisor in a timely manner.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Intervention team
Caregivers, teachers, other service providers, the RBT, and the supervisor.

Concern
A problem someone raises about the learner, the services, or the plan.

Timely
As soon as possible, using the method and timeline your supervisor and organization set.

Discriminative nuances

Relay vs. Decide
The RBT passes the message to the supervisor. The RBT does not decide whether to change the plan.

Timely vs. Later
Safety concerns and changes in the learner are reported right away. Do not wait for the next scheduled meeting.

3 STEP-BY-STEP PROCEDURE

1. **Listen**
   Listen to the concern or suggestion and note it as stated.
2. **Do not decide**
   Do not promise changes or give advice about the plan.
3. **Tell the supervisor**
   Tell the supervisor in a timely manner, using the agreed method.
4. **Close the loop**
   Return to the caregiver or teacher with the supervisor's response, as your supervisor directs.

RBT SCOPE BOUNDARY

RBTs are responsible for relaying concerns and suggestions to the supervisor in a timely manner. The BCBA reviews them and decides on any change to the plan.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

A teacher tells the RBT the learner has been hitting more at school this week. The RBT does not change the plan. The RBT tells the supervisor that day.

Concern passed to the supervisor the same day

NON-EXAMPLE **Common pitfall**

Incorrect: Telling the teacher, "We will stop that program," without asking the supervisor.

Why it's wrong: Changing a program is the supervisor's decision. The RBT relays the concern.

5 · EXAM TIP
High-yield summary

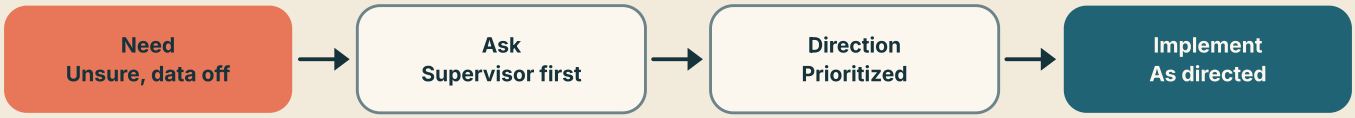
Test takeaway: "Team member raises a concern" points to telling the supervisor in a timely manner.

TASK
E.2

1 TASK E.2 · CLINICAL DIRECTION

Seeking and Prioritizing Clinical Direction

Core definition. Asking the supervisor for direction when needed and putting that direction first, following the chain of command.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Clinical direction
Instructions from the supervisor about services, such as training needs or how to handle a problem.

Chain of command
The order of who to go to, starting with your supervisor.

Data irregularity
Data that look unusual or do not match what was observed, such as a sudden jump.

Discriminative nuances

Seeking vs. Guessing
When unsure, ask the supervisor. Do not guess or change a procedure on your own.

Direction vs. Request
Clinical direction from the supervisor comes before requests from others, unless it is unsafe or unethical.

3 STEP-BY-STEP PROCEDURE

1. **Notice the need**
   Notice a training need, a data irregularity, or a question about a procedure.
2. **Go to your supervisor**
   Ask your supervisor first and follow the chain of command.
3. **Prioritize the direction**
   Follow the supervisor's direction before other requests.
4. **Record it**
   Note the direction you received and when, per policy.

RBT SCOPE BOUNDARY

RBTs are responsible for asking for direction in a timely manner and following it. The BCBA provides direction and training.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

An RBT sees a new prompt level in the plan and is unsure how to run it. The RBT asks the supervisor before the next session and practices it as directed.

Training need raised and addressed before implementing

NON-EXAMPLE **Common pitfall**

Incorrect: Running the procedure the way a coworker described, without asking the supervisor.

Why it's wrong: The supervisor provides direction. Other staff are not the chain of command.

5 · EXAM TIP
High-yield summary

Test takeaway: "Unsure how to implement" points to asking the supervisor first.

TASK
E.3

1 TASK E.3 · DOCUMENTATION

Reporting Variables That Affect Client Progress

Core definition. Reporting events and conditions that might affect the learner's performance to the supervisor in a timely manner.

Illness	Medication	Schedule	Home / school
sick, sleep	new or changed	new routine	big change

Report any of these to the supervisor in a timely manner.

2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Variable
Any event or condition that might change the learner's behavior or performance.

Examples
Illness, medication changes, sleep, schedule changes, and changes at home or school.

Timely
Reported promptly, in the way the supervisor and organization require.

Discriminative nuances

Report vs. Interpret
The RBT reports what was observed or told. The RBT does not conclude what caused a change.

Medication Changes
Report that a change was mentioned. Do not give advice about medication.

3 STEP-BY-STEP PROCEDURE

1. **Notice**
   Notice the variable, or hear about it from a caregiver.
2. **Note it**
   Write down what was said or observed and when.
3. **Report promptly**
   Report it to the supervisor in a timely manner.
4. **Note it with the data**
   Document it with the session data so it can be considered.

RBT SCOPE BOUNDARY
RBTs are responsible for noticing, recording, and reporting variables promptly. The BCBA decides whether and how it affects the plan.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

A parent says the learner started a new medication yesterday. During the session, the learner is tired and responds less. The RBT notes both and tells the supervisor that day.

Variable reported and documented with the data

NON-EXAMPLE **Common pitfall**

Incorrect: Not mentioning a missed night of sleep because the data looked fine.

Why it's wrong: Variables can affect progress. The supervisor decides what matters.

5 · EXAM TIP
High-yield summary

Test takeaway: "Illness, medication, schedule change" points to reporting it in a timely manner.

TASK
E.4

1 TASK E.4 · DOCUMENTATION

Communicating Objectively What Occurred

Core definition. Describing what happened in a session using observable facts, in line with legal, regulatory, and workplace requirements.

Objective	Subjective
Threw cup, 1 min	Was frustrated

Write the left column. Leave out the right column.

2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Objective
Observable and measurable. Another person could confirm it.

Subjective
An opinion, guess, or feeling about what happened or why.

Requirements
Laws, regulations, and workplace policies for documentation, such as privacy and timelines.

Discriminative nuances

Objective vs. Subjective
"Screamed for 2 minutes" is objective. "Was angry" is subjective.

Documenting vs. Interpreting
Describe what happened. Leave out guesses about why.

3 STEP-BY-STEP PROCEDURE

1. **Use observable terms**
   Describe behavior and events in observable terms.
2. **Include the facts**
   Include what happened, what you did, and when.
3. **Follow requirements**
   Follow legal, regulatory, and workplace requirements, including privacy.
4. **Submit on time**
   Complete and submit notes by the required deadline.

RBT SCOPE BOUNDARY

RBTs are responsible for writing objective session notes that follow requirements and are submitted on time. The BCBA reviews notes and interprets the data.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

A session note says, "At 10:15, the learner threw a cup and said 'no' for 1 minute. The RBT removed the cup and said 'Finish first.'" It leaves out why the learner did it.

Objective note a reader can verify

NON-EXAMPLE **Common pitfall**

Incorrect: Writing, "The learner was frustrated and wanted attention."

Why it's wrong: This is a guess about feelings and function. It is not an observable fact.

5 · EXAM TIP
High-yield summary

Test takeaway: "Observable and measurable" points to objective documentation.

DOMAIN

F

Ethics

11 QUESTIONS · 15% OF EXAM

10 TASKS · 10 PROFILES

F.1	Core Principles of the RBT Ethics Code	80
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F.7	Multiple Relationships	86
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F.10	Cultural Humility and Responsiveness	89

TASK
F.1

1 TASK F.1 · ETHICS CODE

Core Principles of the RBT Ethics Code

Core definition. The four core principles behind the RBT Ethics Code (2.0): benefit others; treat others with compassion, dignity, and respect; behave with integrity; and ensure your own competence.

Benefit others	Compassion	Integrity	Own competence
best interest	dignity, respect	honest, accurate	trained, shown

Use all four to check any choice.

2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Benefit others
Act in the best interest of the learner and others, and protect them from harm.

Compassion, dignity, and respect
Treat every learner, family member, and colleague this way, in words and actions.

Integrity
Be honest and follow the code, laws, and policies, even when no one is watching.

Ensure own competence
Work within your competence and keep building it with your supervisor.

Discriminative nuances

Principle vs. Standard
Principles guide choices. Numbered standards apply them.

Ethics vs. Law
The law is required. The code can ask for more.

3 STEP-BY-STEP PROCEDURE

1. **Know the code**
   Know the code's principles and requirements.
2. **Check each choice**
   Ask whether the action benefits the learner, shows respect, and is honest.
3. **Ask when unsure**
   Ask your supervisor before acting when a situation is unclear.
4. **Report concerns**
   Report possible violations as the code and your supervisor direct.

RBT SCOPE BOUNDARY
RBTs are responsible for following the code in all services and asking when unsure. The BCBA provides guidance on ethical questions.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

A learner has a hard session. The RBT speaks calmly, keeps data accurate, and does not leave out a problem behavior from the notes.

Respectful treatment and honest data

NON-EXAMPLE **Common pitfall**

Incorrect: Leaving a behavior out of the note so the session looks better.

Why it's wrong: This violates integrity. Records must be accurate.

5 · EXAM TIP
High-yield summary

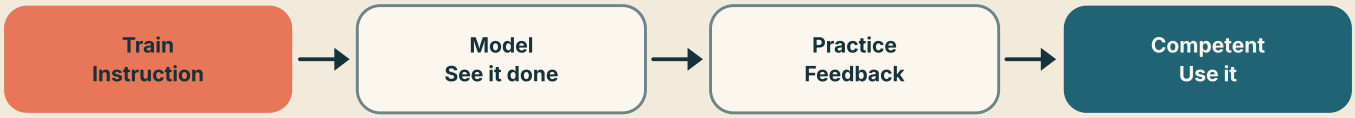
Test takeaway: Benefit others, compassion/dignity/respect, integrity, and own competence are the four principles.

TASK
F.2

1 TASK F.2 · COMPETENCE

Demonstrating Competence Before Providing Services

Core definition. Providing behavior-technician services only after your supervisor confirms you have demonstrated competence.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Competence
The ability to do a task correctly and consistently.

Training
Instruction, modeling, practice, and feedback on a procedure.

Beyond scope
A task outside your certification or competence. Tell your supervisor right away and document it.

Discriminative nuances

Trained vs. Competent
Being told how is not enough. You must show you can do it.

Scope
Services stay within your training, the plan, and your role as an RBT.

3 STEP-BY-STEP PROCEDURE

1. **Get trained**
   Complete training on the procedure before using it.
2. **Show competence**
   Your supervisor confirms you can do it before you provide the service.
3. **Stay in scope**
   Use only procedures you are trained on and that are in the plan.
4. **Tell and document**
   If asked to do something beyond your competence, tell your supervisor right away and document it.

RBT SCOPE BOUNDARY
RBTs are responsible for using only procedures their supervisor has confirmed they are competent in, and reporting anything beyond scope. The BCBA provides training and confirms competence.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

A new plan includes a prompt fading procedure. The RBT tells the supervisor they have not run it. The supervisor models it and watches the RBT practice before the RBT uses it with the learner.

Procedure used after competence shown

NON-EXAMPLE **Common pitfall**

Incorrect: Running a new procedure from a description, without practice or feedback.

Why it's wrong: Competence must be shown before services are provided.

5 · EXAM TIP
High-yield summary

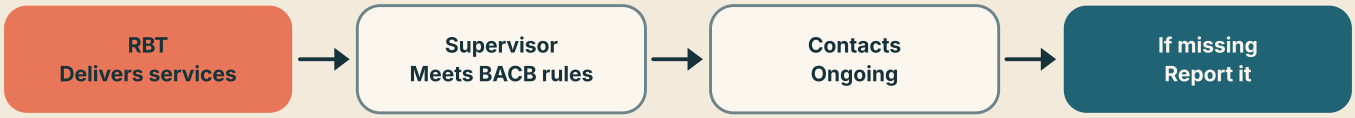
Test takeaway: "New procedure" points to training and showing competence first.

TASK
F.3

1 TASK F.3 · SUPERVISION

Providing Services Under Ongoing Supervision

Core definition. Providing services under the RBT certification only within a clearly defined role, under close, ongoing supervision.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Supervisor
An RBT Supervisor or RBT Requirements Coordinator who meets BACB requirements.

Ongoing supervision
Supervision that continues for as long as the RBT provides services.

Supervision requirement
At least 5% of the hours you provide services each month (check the RBT Handbook for the latest requirements).

Discriminative nuances

Supervision vs. Independence
An RBT does not work independently. The supervisor oversees all services.

Qualified vs. Available
A supervisor must meet BACB requirements. Someone who is simply present does not qualify.

3 STEP-BY-STEP PROCEDURE

1. **Know your supervisor**
   Know who your supervisor is and that they meet BACB requirements.
2. **Attend supervision**
   Attend every scheduled supervision contact.
3. **Report lapses**
   Tell your organization if supervision is missed or your supervisor changes.
4. **Keep records**
   Keep supervision records as required.

RBT SCOPE BOUNDARY
RBTs are responsible for attending supervision and reporting any lapse. The BCBA meets BACB supervision requirements and oversees services.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

An RBT's supervisor leaves the practice. The RBT tells the organization, and services continue only after a qualified supervisor is assigned.

Services continue under a qualified supervisor

NON-EXAMPLE **Common pitfall**

Incorrect: Continuing services with no supervisor for a few weeks because sessions went well.

Why it's wrong: Services require ongoing supervision from a qualified supervisor.

5 · EXAM TIP
High-yield summary

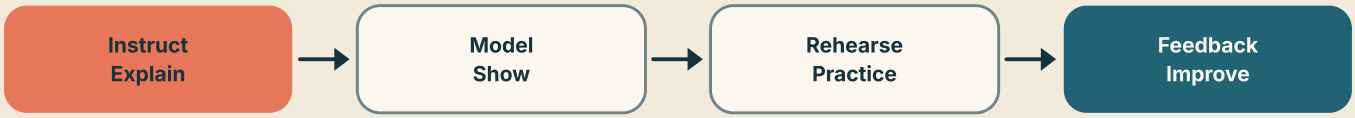
Test takeaway: "No supervisor available" points to reporting it, not continuing alone.

TASK
F.4

1 TASK F.4 · SUPERVISION

Effective Supervision Practices

Core definition. Supervision that includes training with instructions, modeling, rehearsal, and feedback, plus observation of the RBT delivering services.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Instructions, modeling, rehearsal, feedback
The four parts of effective training.

Observation
The supervisor watches the RBT deliver services.

Feedback
Information about your performance that helps you improve.

Discriminative nuances

Observation vs. Meeting
A meeting alone is not enough. Observing service delivery is required.

Feedback vs. Criticism
Feedback is specific and about the work. Accept it and use it.

3 STEP-BY-STEP PROCEDURE

1. **Take part in training**
   Take part in instruction, modeling, and rehearsal.
2. **Welcome observation**
   Allow the supervisor to observe your sessions.
3. **Use feedback**
   Apply the feedback in your next session.
4. **Ask for what you need**
   Ask for training or support when needed.

RBT SCOPE BOUNDARY

RBTs are responsible for participating in supervision, accepting feedback, and applying it. The BCBA plans and provides supervision.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

The supervisor explains a trial, models it, has the RBT practice, and gives feedback. Later the supervisor watches the RBT run it with a learner.

Training followed by observation of service delivery

NON-EXAMPLE **Common pitfall**

Incorrect: Counting a phone call about the schedule as supervision of service delivery.

Why it's wrong: Effective supervision includes observation of the RBT delivering services.

5 · EXAM TIP
High-yield summary

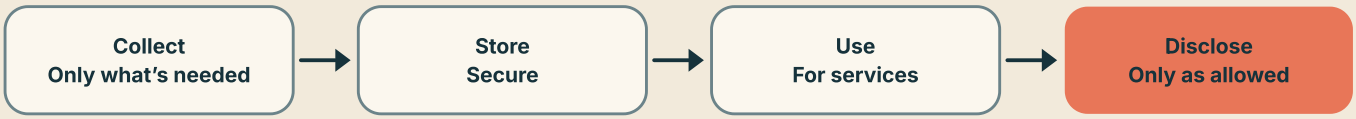
Test takeaway: "Instructions, modeling, rehearsal, feedback" points to effective supervision.

TASK
F.5

1 TASK F.5 · CONFIDENTIALITY

Confidential Information

Core definition. Following requirements for collecting, using, storing, protecting, and disclosing information about learners and families.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Confidential information
Information about a learner or family, such as name, diagnosis, data, and records.

Disclosure
Sharing information with someone else. Do so only as allowed.

Requirements
Laws, the code, and workplace policies, such as HIPAA, that govern the information.

Discriminative nuances

Need to Know vs. Interest
Share only with those who need it for services and are allowed to receive it.

Storing vs. Using
Protect information in every form, such as paper, devices, and messages.

3 STEP-BY-STEP PROCEDURE

1. **Collect only what is needed**
   Collect only what the services need.
2. **Store it safely**
   Keep records secured and devices protected.
3. **Share as allowed**
   Discuss client information only under your supervisor's direction, unless the law allows it. Share only what is necessary.
4. **Report breaches**
   Report lost or shared information to your supervisor right away.

RBT SCOPE BOUNDARY

RBTs are responsible for protecting learner information and sharing it only as allowed. The BCBA sets and enforces the practice's confidentiality procedures.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

At a store, a neighbor asks how a learner is doing. The RBT says they cannot discuss clients and ends the conversation.

Information not disclosed

NON-EXAMPLE **Common pitfall**

Incorrect: Texting a coworker a learner's full name and data over a personal phone.

Why it's wrong: Personal messages are not protected. Information is shared only as allowed.

5 · EXAM TIP
High-yield summary

Test takeaway: "Share only as allowed" points to confidentiality.

TASK
F.6

1 TASK F.6 · PUBLIC STATEMENTS

Public Statements and Social Media

Core definition. Making only accurate statements about your credentials and services, and never sharing identifying client information on social media or websites.

Allowed	Not allowed
Your own accurate credential	Client photo, video, details

When unsure, do not post. Ask your supervisor.

2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Public statement
Anything you post or say publicly, such as social media, websites, or advertisements.

Identifying information
Photos, videos, or written details that could identify a client.

Misrepresentation
A false, misleading, or exaggerated statement about your qualifications or services.

Discriminative nuances

Credentials vs. Claims
State your RBT certification accurately. Do not claim titles or abilities you do not have, such as BCBA.

Blurred vs. Identifying
Hiding a name does not make a post safe if the client can still be identified.

3 STEP-BY-STEP PROCEDURE

1. **Check before posting**
   Ask whether the post shares client information or describes your work.
2. **Keep clients out**
   Never post photos, videos, or details about clients.
3. **Be accurate**
   Describe your credentials and services accurately.
4. **Ask when unsure**
   Ask your supervisor and follow your employer's policy.

RBT SCOPE BOUNDARY

RBTs are responsible for keeping client information off social media and websites and describing their credentials accurately. The BCBA sets and explains the practice's policies.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

An RBT has a rewarding session. A friend asks to see a photo of the learner's artwork with the learner's name. The RBT does not share it or post about the session.

No client information shared online

NON-EXAMPLE **Common pitfall**

Incorrect: Posting, "Best RBT ever, I cured a child's tantrums," with the client's first name hidden.

Why it's wrong: This exaggerates services and can still identify the client.

5 · EXAM TIP
High-yield summary

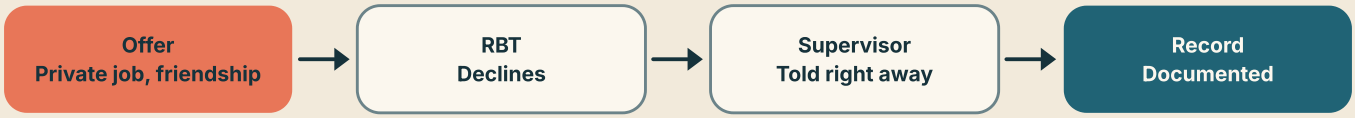
Test takeaway: "Identifying information on social media" points to a violation, even with the name removed.

TASK
F.7

1 TASK F.7 · MULTIPLE RELATIONSHIPS

Multiple Relationships

Core definition. Mixing two or more roles, such as professional and personal, with a client, stakeholder, supervisor, or coworker in a way that can create conflicts of interest and risk of harm.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Multiple relationship
A mix of an RBT's roles with the same person, such as friend and RBT.

Conflict of interest
A personal, financial, or professional interest that can affect professional judgment.

Avoid
Prevent the relationship from forming where possible.

Discriminative nuances

Avoid vs. Report
Avoid multiple relationships. If one develops, tell your supervisor right away.

Supervisor Involved
If the relationship involves your supervisor, report it to your supervisor's manager or another appropriate entity, such as human resources.

3 STEP-BY-STEP PROCEDURE

1. **Spot the risk**
   Notice when a client or family member asks to be a friend or to hire you privately.
2. **Decline politely**
   Keep to your professional role.
3. **Tell your supervisor**
   Tell your supervisor right away if one has developed.
4. **Document**
   Document your actions and follow the plan to resolve it.

RBT SCOPE BOUNDARY
RBTs are responsible for avoiding multiple relationships, telling their supervisor right away if one develops, and documenting. The BCBA helps resolve it.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

A parent offers to pay an RBT to babysit the learner on weekends. The RBT declines and tells the supervisor about the offer.

Role kept professional and supervisor informed

NON-EXAMPLE **Common pitfall**

Incorrect: Accepting the babysitting job because the learner likes you and the pay is good.

Why it's wrong: This mixes an employee and a personal-service role with a client's family and risks a conflict of interest.

5 · EXAM TIP
High-yield summary

Test takeaway: "Friend, family, or employer of the client" points to a multiple relationship.

TASK
F.8

1 TASK F.8 · GIFTS

Giving and Receiving Gifts

Core definition. Not giving or accepting gifts with a monetary value over $10 US from clients, stakeholders, or supervisors, and following stricter employer policies.

Up to $10	Over $10	Stricter policy
Occasional thanks	Decline	Follow employer

The employer's stricter policy always wins.

2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Monetary value limit
$10 US or the equivalent purchasing power in another currency.

Acceptable gift
An occasional expression of gratitude that gives no financial benefit to the recipient.

Cumulative gifts
Ongoing or repeated gifts that become a regularly expected source of income or value can violate the standard.

Discriminative nuances

Code vs. Employer
If your employer's policy is stricter, such as no gifts at all, follow it.

Gift vs. Payment
Gifts are not payment for services. Do not expect them.

3 STEP-BY-STEP PROCEDURE

1. **Check the value**
   Estimate the value before accepting or giving.
2. **Check the policy**
   Check your employer's policy.
3. **Decline kindly**
   Politely decline gifts that are over the limit or against policy.
4. **Tell your supervisor**
   Tell your supervisor about any gift question.

RBT SCOPE BOUNDARY

RBTs are responsible for following the gift limit and employer policy and telling their supervisor about gift questions. The BCBA answers questions about policy.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

A family gives the RBT a $5 thank-you card and small candy at the holidays. The RBT accepts, because it is occasional and under $10, and mentions it to the supervisor.

Small, occasional gift acceptable

NON-EXAMPLE **Common pitfall**

Incorrect: Accepting a $50 gift card from a family.

Why it's wrong: The value is over $10.

5 · EXAM TIP
High-yield summary

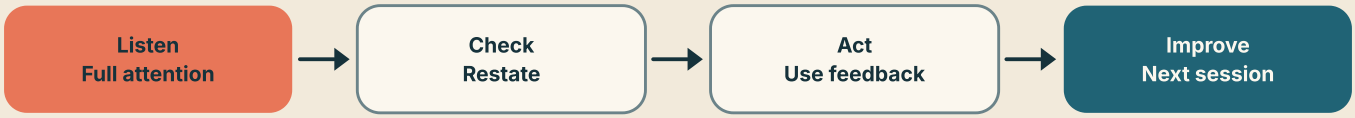
Test takeaway: "Over $10" points to declining, unless the employer's policy is stricter.

TASK
F.9

1 TASK F.9 · PROFESSIONAL SKILLS

Interpersonal and Professional Skills

Core definition. Using skills such as accepting feedback, listening actively, seeking input, and collaborating when representing yourself as an RBT.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Accepting feedback
Listening to supervisor feedback and acting to improve.

Active listening
Giving your full attention and checking that you understood.

Collaboration
Working with the supervisor, caregivers, and other providers toward the learner's goals.

Discriminative nuances

Feedback vs. Defending
Accepting feedback means improving your performance, not arguing against it.

Seeking vs. Guessing
Seek input from your supervisor when you are unsure.

3 STEP-BY-STEP PROCEDURE

1. **Listen**
   Listen without interrupting.
2. **Check understanding**
   Restate what you heard and ask questions.
3. **Act on it**
   Use the feedback in your next session.
4. **Stay professional**
   Stay professional in all work activities.

RBT SCOPE BOUNDARY
RBTs are responsible for behaving professionally, accepting feedback, and acting on it. The BCBA provides feedback and support.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

After a session, the supervisor says the RBT delivered prompts too late. The RBT says, "Thanks, I'll practice that," and uses a prompt timer in the next session.

Feedback accepted and used

NON-EXAMPLE **Common pitfall**

Incorrect: Responding, "That's how the last supervisor taught me," and not changing.

Why it's wrong: Professional conduct means acting to improve after feedback.

5 · EXAM TIP
High-yield summary

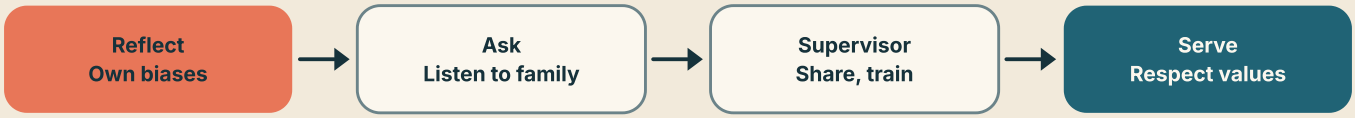
Test takeaway: "Respond to feedback" points to acting on it professionally.

TASK
F.10

1 TASK F.10 · CULTURAL RESPONSIVENESS

Cultural Humility and Responsiveness

Core definition. Working with your supervisor to be culturally responsive, evaluating your own biases, and treating everyone in an equitable and inclusive way.



2 KEY CONCEPTS & CLINICAL TERMINOLOGY

Essential terms

Cultural humility
Ongoing self-reflection on your own biases and openness to learn from others.

Bias
An assumption or preference that can affect how you treat someone.

Diverse needs and backgrounds
Differences such as age, disability, ethnicity, language, religion, and socioeconomic status.

Discriminative nuances

Humility vs. Expertise
You do not need to know everything about a culture. You listen, ask, and learn.

Bias vs. Discrimination
You work on your biases. The code prohibits harassment and discrimination.

3 STEP-BY-STEP PROCEDURE

1. **Reflect**
   Identify your own biases and assumptions.
2. **Ask and listen**
   Ask families about their values, routines, and preferences.
3. **Work with your supervisor**
   Share what you learn and get training as needed.
4. **Treat everyone equitably**
   Treat all clients and coworkers equitably and inclusively.

RBT SCOPE BOUNDARY
RBTs are responsible for reflecting on their own biases, listening to families, and treating everyone equitably. The BCBA guides cultural responsiveness in the plan.

4 REAL-WORLD SCENARIO & APPLIED EXAMPLES

EXAMPLE **Clinical example**

A family does not eat at the table during meal programming. The RBT asks about their routines, shares it with the supervisor, and the plan is adjusted.

Plan fits the family's values

NON-EXAMPLE **Common pitfall**

Incorrect: Telling the family their routine is wrong and that they should do it the RBT's way.

Why it's wrong: This ignores the family's values. Ask and work with the supervisor.

5 · EXAM TIP
High-yield summary

Test takeaway: "Self-reflection on bias" and "respect for the family's values" point to cultural humility.